

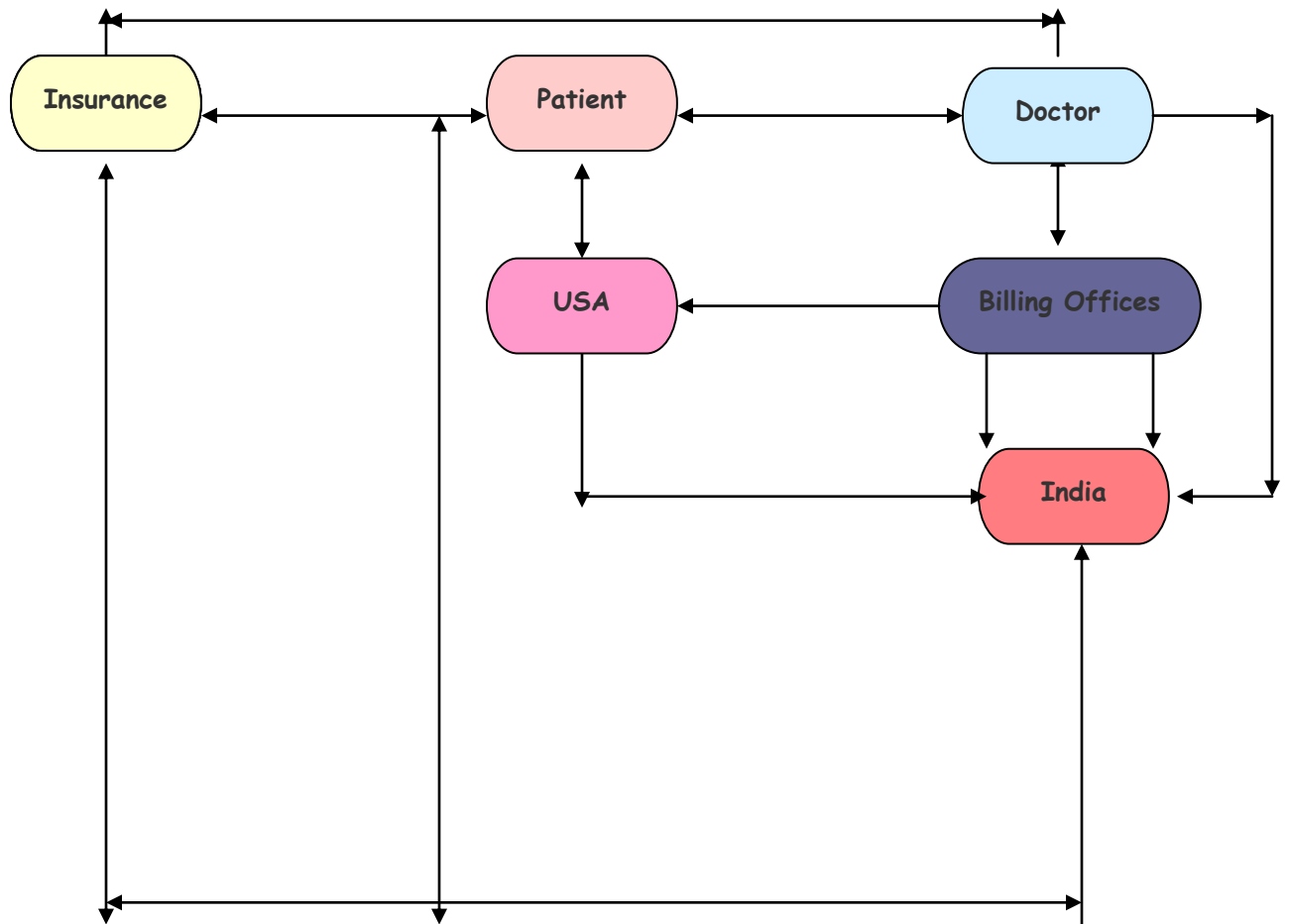
MEDICAL BILLING



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Medical Billing - OUTSOURCING



Medical Billing - WHAT IS IT?



In a NUTSHELL - Medical billing (United States):

Definition:

Medical Billing is the practice of submitting claims to Insurance companies (Carrier) either Private or Federal (United States Government), specifically Medicare, in order to receive payment for services provided to a patient by the Doctor (Physician / Provider).

This process is typically performed in a series of steps

- The Doctor examines a patient who is Sick, Injured or requires Health Management.
- Depending on the examination & service provided, the doctor creates or updates the patient's medical record.
- This record will contain information given by the patient to the Doctor / Doctor's office regarding illnesses, injuries or lifestyle prior to the treatment. This forms the basis for the diagnosis and the corresponding treatment derived from it.
- The treatment, along with the diagnosis, and even the time spent with a patient are combined to determine the level of service or procedure provided to the patient.
- This information is then sent by the Doctor's Office to the Medical Transcriptionist in the form of voice files.

- The next step is to transcribe these files into data files. This is done by a Medical Transcriptionist, and this process is termed as Medical Transcription
- These data files are then coded by a Medical Coder (A certified professional coder is known as a CPC). Medical Coding is the process of converting the diagnostic and procedural descriptions (data files) into a set of standard Alpha / Numeric codes, which are determined & periodically updated by the American Medical Association. The diagnostic records are converted to ICD codes - International Classification of diseases & the treatment or procedure records are converted to CPT codes - Current Procedural Terminology. Medical coding ensures accurate billing and reimbursement
- Once the medical records are coded they are sent to the Billing Office. The billing office is the place where charges are created for the doctor's services & sent to respective insurance companies for payment. The billing office works on behalf of the doctor
- Next in the Medical Billing Office, in the Charge Entry Department a billing record, either paper (usually a standardized form called an HCFA / UB92) or electronic, is generated based on the information provided by the coder. This billing record or claim is then submitted either to a clearinghouse that acts as an intermediary for the information (this is typical for electronic records) or directly to the insurance company for payment. This process is termed as Claims Transmission
- The claim is processed by the respective insurance company. It is then either paid or denied. This explanation is sent in the form of an EOB (Explanation of Benefits) to the billing office
- In the billing office, Cash Application / Posting is done to those accounts for which payment is received

- Claims denied for payment, are sent to the AR (Accounts Receivable) department. Here the reasons for denial are analyzed by analysts & calls to the insurance are made by callers, to sort issues & receive payment

Medical Billing - WHY IS IT DONE?



"I've finished the treatment - it's his wallet I'm having trouble extracting!"

Concept

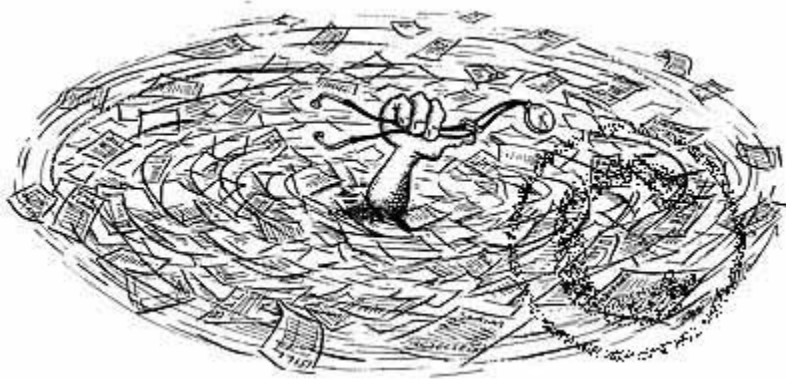
In the present day physicians are not paid immediately by the patient for the services rendered. Instead in most cases, the Insurance Company makes the payment to the physician on behalf of that patient, if he/she has health insurance coverage with them.

In the United States majority of the patients have insurance coverage and details of such coverage are provided to the physician before treatment. It is the responsibility of the physician to submit claims to the insurance company and get paid for his services.

Submitting Claims and getting paid is a lengthy process and involves a lot of rules and regulations and is very complicated. The physician needs to adhere to all these rules before submitting claims. This process is very diverse from the physician's core activity & calls for a particular set of skills. It is usually outsourced to a set of professionals called Medical Billers. This activity paves the way for the concept of Medical Billing.

Healthcare has become one of the fastest growing industries in the United States. This growth can be attributed to the recent legislation and changes in medical insurance. Hospitals, private practices and clinics could not survive without someone to efficiently handle their day-to-day medical billing procedures.

Medical Billing - FOR WHO IS IT DONE?



Doctor drowning

Medical Billing is done for the Doctor. The process of creating a charge for the services the doctor renders and claiming the same from the insurance is outsourced to a Billing office. This helps save time & energy for the doctor & allows him/her to concentrate more on his/her core area of business - Health Care Management.

Medical Billing - WHO DOES IT?

Responsibilities of a Billing Office

- Coding
- Maintaining patient's records and accounts
- Filing claims to the insurance companies
- Following up with the insurance companies and patients
- Assuring high productivity and re-imbursement
- Strict adherence to rules and laws of insurance companies



There are three parties involved in the process of Medical Billing.

- The PHYSICIAN.
- The 3rd PARTY INSURANCE COMPANY
- The PATIENT.

The billing office on behalf of the Provider does the actual process of Billing or Insurance Claiming. Physicians appoint Medical Billing Companies to take care of their billing. The Service Level Agreement between the Billing Company & the Physician clearly determines the duties, responsibilities & the process to be adopted by the Billing Office in carrying out the objectives of the Physician. The main objective of the Billing Company is to maximize collections for the Physician.

Medical Billing - WHO IS BILLED?

Patients take policies with a health insurance company, to cover their health care costs or liabilities. The insurance company takes responsibility of all the financial risks (within the scope of the policy) undergone by the patient, in relation to medical treatment for himself or his dependents during the tenure of the policy. Therefore usually the charges are paid by the Insurance Company & very rarely are borne by the Patients.



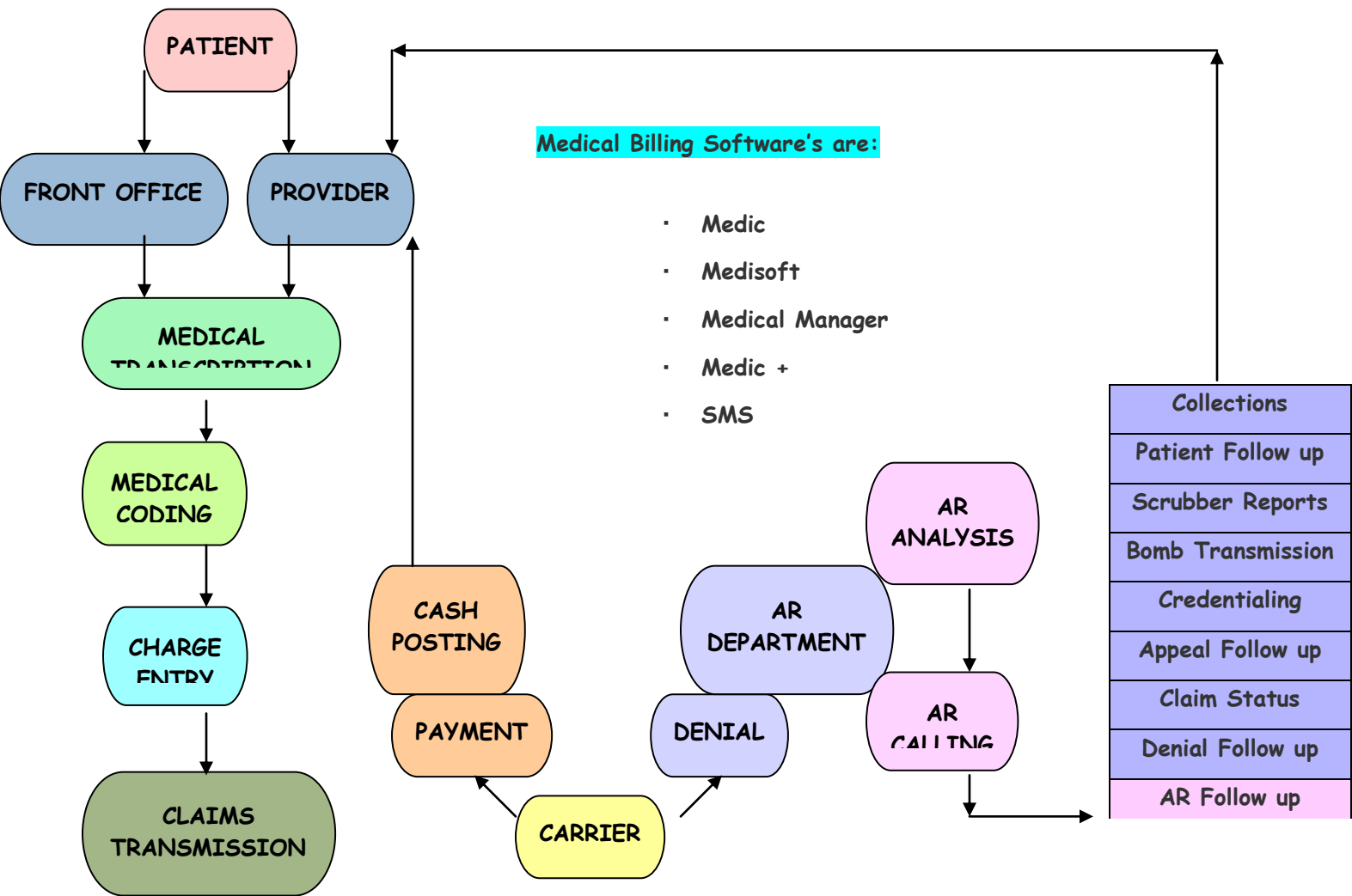
Medical Billing - HOW IS IT DONE?

For many decades, Medical Billing was done almost entirely on paper. However, with the advent of computers it became possible to manage more claims at a time. This eventually brought about the business of medical billing.

Responsibilities involve accurately completing claim forms, promptly billing insurance companies, and adhering to each insurance carrier's policies and procedures.

The Process

Medical Billing Workflow



Insurance

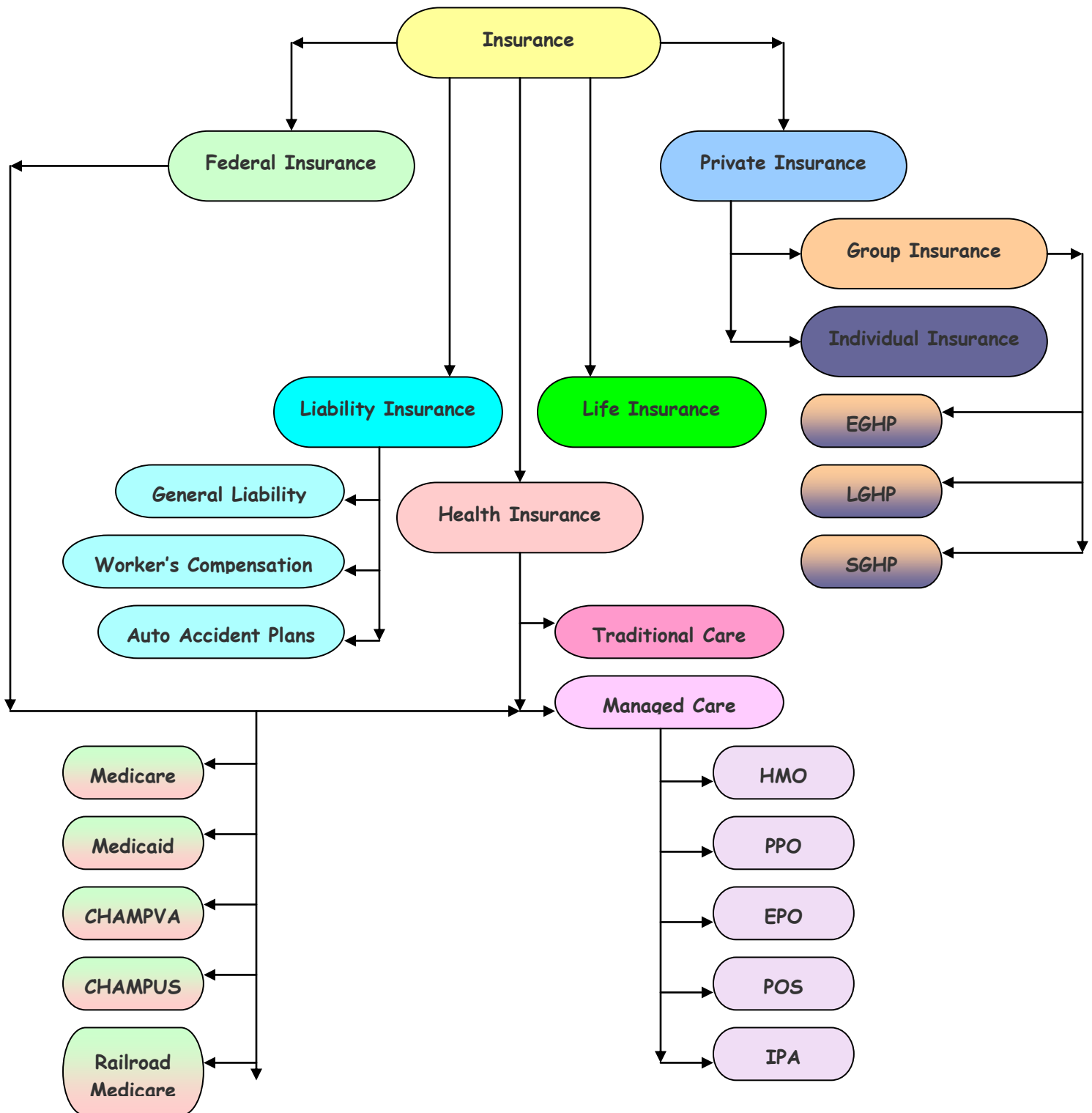
What is Insurance?

Insurance is a contract that exists between an organization & a person, to protect the person from the losses or damage, that he might incur on account of the particular risk or hazard, that has been stated therein in the contract. The person giving the assurance is called the **Insurer** and the person who is receiving the protection is called the **Insured**.

Definition: A contract that provides compensation for specific losses in exchange for a periodic payment. An individual contract is known as an insurance policy, and the periodic payment is known as an insurance premium.

Examples of the different types of insurance available are automobile, home, health and worker's compensation. Whereas in most cases the insured is paid for their loss, with life insurance a beneficiary is paid when the insured person passes away.

INSURANCE OVERVIEW



Types of Insurance Providers

There are 2 types of insurance companies:

- Public Insurance
- Private Insurance

Public Insurance Companies

Public Insurance Companies are otherwise known as Federal Insurance Companies. They are owned by the Federal Government. The insurer is the Government.

Private Insurance Organizations

These are privately owned & run organizations.

They are divided into:

- Individual Insurance
- Group Insurance
- Individual Insurances Policies

They are insurances that give the subscriber control over the policy, premium & coverage.

- Group Insurances Policies

They are insurances that are applicable to a group of people. No individual subscriber has control over the policy, premium & coverage.

Group coverage's are divided into:

- EGHP (Employee Group Health Programs that covers a group of 20 - 100 people
- LGHP (Large Group Health Programs) that covers a group of more than 100 people

- SGHP (Small Group Health Programs) that covers a group of less than 20 people.



Types of Insurance Coverage

Any risk that can be quantified probably has a type of insurance to protect it.

They can be broadly classified into 3 types:

- Life Insurance
- Liability Insurance
- Health Insurance

The different types of insurance are:

- Life insurance provides a benefit to a decedent's family or other designated beneficiary, to replace loss of the insured's income and provide for burial and other final expenses.
- Annuities provide a stream of payments and are generally classified as insurance because they are issued by insurance companies and regulated as insurance. In a sense, they are the opposite of life insurance.
- Health insurance covers medical bills incurred because of sickness / injury or accidents.
- Liability insurance covers legal claims against the insured. For example, a doctor may purchase insurance to cover any legal claims against him if he were to be convicted of a mistake in treating a patient.
- Automobile insurance, also known as auto insurance, car insurance and in the UK as motor insurance, is probably the most common form of insurance and may cover both legal liability claims against the driver and loss of or damage to the vehicle itself.
- Property insurance provides protection against risks to property, such as fire, theft or weather damage. This includes specialized forms of insurance such as fire insurance, flood insurance, earthquake insurance, home insurance or boiler insurance.

- Casualty insurance insures against accidents, not necessarily tied to any specific piece of property.
- Financial loss insurance protects individuals and companies against various financial risks. For example, a business might purchase cover to protect it from loss of sales if a fire in a factory prevented it from carrying out its business for a time. Insurance might also cover failure of a creditor to pay money it owes to the insured. Fidelity bonds and surety bonds are included in this category.
- Title insurance provides a guarantee on research done on public records affecting title to real property, usually in conjunction with a search done at the time of a real estate transaction, such as a sale, or a mortgage.
- Credit insurance pays some or all of a loan back when certain things happen to the borrower like unemployment, disability, or death.
- Terrorism insurance covers liabilities that arise out of acts of terrorism.
- Political risk insurance can be taken out by businesses with operations in countries in which there is a risk that revolution or other political conditions will result in a loss.
- Worker's Compensation insurance replaces all or part of a worker's wages and accompanying medical expense lost due to a job-related accident / injury.

Liability Insurance

They are of 3 types:

- General Liability
- Worker's Compensation
- Auto accident Plans

General Liability insurances

They generally cover liabilities arising out of fire, theft, marine & natural calamities.

They also provide house owner's insurance & shopkeepers insurance.

Worker's Compensation

They cover liabilities that occur due to accidents in the work place.

Features:

- Employer insures all employee of the organization thus reducing the risk of huge loss
- Employer pays premium
- Employee is the subscriber
- Claims are filed along with, injury report, ombudsmen report - report of the government, witness report, provider's note, employer's note
- Claims are sent in C4 forms
- Claims are usually on paper
- Payments are made in lump sum amounts
- Insurance company, employer & the employee play an equal role.
- WC Insurances are provided by Private Insurances, Public - Federal, State Insurances & by the Employer
- OWCP - Office of worker's compensation program ensure that employees get a fair compensation. They identify risk prone areas & formulate suitable regulations

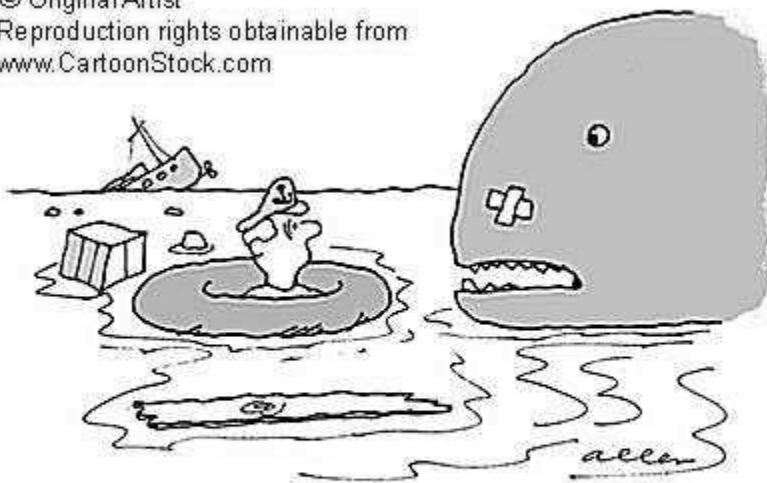
Auto Accident Plans

They cover liabilities that occur due to accidents involving vehicles.

Features:

- Personal Injury Protection (PIP) SELF
- Collision Coverage (Self's Property/vehicle)
- Bodily Injury Liability (Damage to the other party)
- Property Damage Liability (Damage to the other party's property/vehicle)
- Comprehensive Coverage (Damages that occur to the vehicle due to events like - storm, rioting etc)
- Uninsured Motorists Coverage (Protection against damages if the other party involved in the accident is uninsured)
- No Fault Liability (Protection against accidents, irrespective of which party is at fault)

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"Sorry about your boat. Who do I see about personal injury compensation?"

Health Insurance

What is Health Insurance?



Introduction

In insurance, the insured makes payments called "premiums" to an insurer, and in return is able to claim a payment from the insurer if the insured suffers a defined type of loss. This relationship is usually drawn up in a formal legal contract, also known as a policy. The contract will set out in detail the exact circumstances under which a benefit payment will be made and the amount of the premiums.

For example, many individual people purchase health insurance policies and they each pay a small monthly or yearly premium to an insurance company. When a policyholder gets ill, the insurance company provides money to cover medical treatment. For some individuals the insurance benefits may total far more money than they have ever paid into the insurance policy. Others may never make a claim. When averaged out over all of the people buying policies, value of the claims even out. Insurance companies set their premiums based on their calculated payouts.

Key Features

- Health insurance is a protection against health / medical related costs or damages.
- Health insurance policy is a contract between an insurer and an individual or group, in which the insurer agrees to provide specified health insurance coverage to the insured, for an agreed fee (known as the premium).
- A person insured for health, means he is under Medical Insurance coverage

We need health insurance -

Gone are the days when the physician accepted cash from all the patients directly, immediately after the services were rendered, for the entire amount due for his services. In the past medical service was affordable. But in the present scenario costs of medical services are so high & mostly unaffordable to the middle & lower income group. Here is where the insurance comes into picture.



Unique Features of American Health Industry

- Percentage of insured as compared to other countries is very high. More than 95% of the population has health insurance coverage
- High scope of coverage.
- Availability of a wide range of Insurance plans. Approximately 30,000 plans are available
- Apart from Physicians, unconventional groups such as the carriers, transcriptionist, coders & the billing office are part of the health care system
- Standardization of health care practices - processes, documentations, diagnoses, treatments etc.
- Access to more than one insurance coverage per person

Role of Medical Insurance



"It hurts when I do this, doctor."

- Manages high health care costs
- Helps standardise health care thus improving the quality
- Protects against risk of medical emergencies & uneventful expenditure.
- Gives coverage against terminally ill diseases
- Makes routine conditions requiring frequent hospital visits less expensive
- Preventive health care management, a dream for most becomes a reality
- Enables an organized approach to render health services
- Prevents exploitation by providers
- Improves of overall health

Differences between American and Indian Health care Industry

Indian Scenario

In India, we go to the doctor or Hospital for treatment and settle the bills predominantly by cash.



US Scenario

In the US, people go to the Doctor or Hospital for treatment but do not pay by cash. Instead their insurance company settles their medical bills.

The government takes care of its older citizens by paying them pension thru the Social Security Act and also provides health coverage.



India	USA
Negligible % of people insured	Large % of people insured (more than 95% of the population is insured)
Scope of coverage of plan is limited (pay first and claim later)	Scope of coverage is high
Filing of claims with the insurer is the responsibility of the insured	Filing of claims with the insurer is the responsibility of the providers

Types of Health Insurance

There are two types of health insurance:

- Traditional Care
- Managed Care

Traditional Care Insurance

Traditional Insurance coverage is a contract that exists between the insured and the insurer with an understanding that the insurer will protect the insured from any healthcare cost.

Salient Features of Traditional Care

- The reimbursement by the insurer is a fixed % / fixed amount
- 1. Patient liability is not fixed.
- Insured have higher liabilities.
- 1. Preventive medicine is optional
- No restrictions imposed on the enroller
- No control over providers.
- Higher freedom of choice for patients.
- Patient liability is not fixed.
- Limited Coverage.

Components of Traditional Care

- Basic Coverage - This covers all kinds of emergency services, inpatient services & some outpatient services.
- Major Medical Coverage - This covers other supplementary uncovered and outpatient services
- Comprehensive Coverage - This includes both Basic and Major Medical Coverage

Managed Care Insurance



In this type of coverage the insurance company plays an active role in facilitating the insured's health care activities. This in other words refers to a 3P program where the provider, patient and the 3rd party administrator play an important role.

Salient Features of Managed Care Plans

- Insured's liability is a pre - fixed amt
- Insurer's liability depends on the allowed amount for respective billed amount which varies from carrier to carrier
- Insured is given a network of providers
- Participating provider most often renders the service
- Referral /authorization number from the PCP is required, when seeing a specialist (except for an obgyn)
- Focus on preventive medicine is high

Types of Managed Care Plans



- Health Maintenance Organization (HMO)
- Preferred Provider Organization (PPO)
- Exclusive Provider Organization (EPO)
- Point of service (POS)
- Independent or Individual Practitioner's Association (IPA)

Health Maintenance Organization (HMO)



In a HMO plan a patient has to go through a group of network providers only. Patient stands ineligible for coverage if treated by an out of network provide. Patient needs to visit a PCP (Gate Keeper) first. If specialized care is necessary needs to get a referral / authorization from the PCP. A Co pay or a pre-fixed amount is charged to the patients for every service provided. This amount varies from \$5 to \$25 only.

Salient Features

- Only in network benefits
- Treatment by out of network providers is ineligible. Considered only in the case of emergencies
- Co pay is applicable for all services
- Monthly premium to be paid by the insured
- In HMO 100% of the allowed amount is paid, if the provider is participatory
- PCP is chosen by the patient & all health care activities are managed by him
- Referral / Authorization is mandatory for all services, other than those provided by the PCP
- No Deductible is charged

Preferred Provider Organization (PPO)

In a PPO plan the patient has both in-network benefits and out of network benefits. Patient can visit any provider of his choice. It's not necessary for the rendering provider to be in network. The payment structure varies for in-network and out of network providers. A Deductible which is a pre-fixed amount is charged apart from the premium. The deductible can be paid as a monthly, bimonthly, quarterly, half-yearly or an annual amount. Reimbursements are done only if the deductible is paid for that particular service period.

Salient Features

- Both in network and out of network benefits
- Patient liability is higher in case of out of network services
- The payment structure is not 100%, it is 80% for an in network provider and 60%-70% for an out of network provider.
- Deductible is applicable. Reimbursements done only if deductible is paid
- Premium to be paid apart from the deductible
- No co pay
- Referral / Authorization is not mandatory
- The geographical location of an out of network provider is restricted within a city or certain area.

Exclusive Provider Organization (EPO)



An EPO plan is one in which, a group of providers buy exclusively, HMO policies from insurance companies and they themselves act as the insurance company. The EPO plan is governed by all rules applicable to that HMO plan.

Point of service (POS)

POS plan is a combination of HMO and PPO plans. The difference between PPO and POS is that, in the POS there are no geographical restrictions (i.e Patient can see any provider in any state or city). POS offers the combined advantage of both HMO & PPO plans.

Salient Features

- Self Referral by patient is allowed
- Insured have the flexibility to decide on the type of service
- In case of non-participating provider, the insured has to pay deductible or 30% of the allowed amount (only 70% of the allowed amount is paid by the carrier)
- Premium is higher than HMO & PPO
- In case of out of network provider, service considered as per PPO plan & Deductible is applicable
- In case of in network provider, service considered as per HMO plan & Co-Pay is Applicable.

Independent or Individual Practitioner's Association (IPA)

An IPA plan is one in which, a group of providers buy all types of policies from insurance companies and they themselves act as the insurance company. The IPO plan is governed by all rules applicable to HMO, PPO & POS plans.

This plan is advantageous to health organizations for administrative & business reasons.

Public Health Insurance Companies

Public Insurance Companies are otherwise known as **Federal Insurance Companies**. They are owned by the Federal Government. The insurer is the Government.

List of US Federal Insurance Organizations:

- Medicare
- Medicaid
- Champva
- Champus
- Railroad Medicare

Medicare



Basic Eligibility for coverage under Medicare:

- Citizen of the United States.
- Legal Alien.
- 65 or under 65 years of age.
- Over 65 years and disabled.
- 65 or under 65 years and ESRD patients (End stage Renal Disorders). This act was implemented from 1972.
- You are receiving retirement benefits from Social Security or the Railroad Retirement Board.
- You are eligible to receive Social Security or Railroad benefits but have not yet filed for them.
- You or Your spouse have Medicare-covered Government employment.
- You are entitled to Social Security or Railroad Retirement Board disability benefits for 24 months.
- You are a kidney dialysis or kidney transplant patient.

Medicare has 3 parts

Part-A : Medicare Part A or Hospital Insurance helps cover your inpatient care in hospitals, critical access hospitals and skilled nursing facilities. It also covers hospice care and some home health care.

Part-B : Medicare Part B or Medical Insurance helps cover your doctor's services, outpatient hospital care, and some other medical services that Part A does not cover, such as some of the services of physical and occupational therapists, and some home health care. Part B helps pay for those covered services and supplies when they are medically necessary.

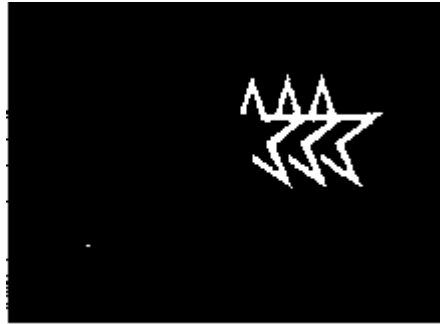
Part-C : Medicare + Choice Plans. Medicare Part C offers all Medicare benefits along with benefits of another plan of your choice. The choice is to be made from a list of plans provided by Medicare.

- **Medicaid**

Features of Medicaid:

- Medicaid is a state-funded health care assistance program for the lower income group and disabled persons. Medical bills are paid from federal, state and local tax funds.
- For Citizens of the United States or legal aliens.
- All rules are state governed & differ from state to state.
- No patient liability.(Sometimes a small co-payment is required.)
- Covers lower income group people, below the state's poverty line of all ages.
- A person is below the poverty line if his total property taken together is below \$4000 for an individual and \$6000 for a family.
- Medicaid cards are issued by HCFA and are renewed every month.

- **Champus**



Champus stands for Civilian health and medical programme for uniformed services.
This coverage is for uniformed services personnel.

- **Champva**



Champva stands for Civilian health and medical programme for veteran administration. It covers people retired from uniformed services.

- **Railroad Medicare**

Covers those who work for the railways department.

Health Insurance Processes

Health Insurance Processes Related to the Provider

Claim Adjudication:

Adjudication is the process of processing claims for either payment or denial by the Utilization Review Department of the insurance company.

The Carrier's Utilization Review Department, reviews the claim and after verification, the claim is adjudicated or processed for payment / denial. The check and an / or Explanation of Benefits are sent to the provider / billing office.

Features of the Utilization Review Department:

1. Insurance companies use automated claims processing which provide them with better record retention and immediate access to a participant's entire claims history.
2. Insurance companies prefer claims submission by fax and they provide toll free fax lines for that purpose, all claims and documentation received by any means are date-stamped, numbered, scanned and electronically logged on to the participant's file.
3. Insurance companies also prefer claims submission electronically and few prefer paper claims.
4. Properly completed claims are automatically adjudicated within 48 hours of receipt and payment is released according to the client's reimbursement schedule.

Utilization Management Department:

- Managed care plans define Utilization Management (UM) as getting the patient the right services, at the right time, for the right price. Since the decision to approve or deny a service can have a significant effect on a patient's medical outcomes.
- UM departments are typically staffed with nurses of backgrounds in various clinical specialties and training in case management techniques. A medical director usually oversees the UM operation. He or she is responsible for reviewing problem cases, making determinations of medical necessity (deciding whether a service was needed to avoid death, disability or worsening of a medical condition) and monitoring plan quality programs.
- In the course of the referral process the PCP sends clinical information to the specialist or testing facility and also notifies the managed care plan that a referral has been authorized. Many plans require the PCP to specify the number of allowed visits or an authorized time period for the referral. For example, the PCP might authorize 10 physical therapy visits or a referral that is valid for one month.
- The patient is responsible for obtaining a referral from the PCP. Specialists can legitimately refuse to see or bill patients who come to a visit without a referral. The managed care plan enters referral authorizations into its UM computer system. This information is stored until a claim is received. The claim is only paid if the service delivered matches the service authorized by the PCP.
- Pre Certification - It is the plan authorization for a hospital stay or an expensive procedure. Typically, a physician or an office staff member calls the plan's Utilization Management Department to report a planned hospitalization. The phone number for this department is usually printed on the member's card. Using plan guidelines (which are often computerized), plan UM staffs approve or deny the admission. As part of the approval process an authorized length of stay (LOS) is assigned. The LOS is number of days the patient is expected to remain in the hospital. If the hospital stay exceeds the number of days authorized, further approval must be sought from the plan.

- Prior authorization - The process of seeking advance approval for expensive or complex testing or treatment. A physician or staff member calls the Utilization Management department of the managed care organization to request authorization of a test or treatment. After reviewing the patient's history and the physician's findings, plan clinical staffs either approve and authorize the requested treatment or deny coverage. An authorization number (a unique number provided by the plan UM Department to verify that the service was authorized) may be provided to the medical practitioner if the service is approved. The authorization number must be placed on the claim submitted by the provider so the claims processing department knows that the services was properly authorized. Prior authorization may also be used in other circumstances such as approval of visits to a non-participating provider.
- Emergency Room Visit authorization- Plans are required to waive the ER authorization rule in cases of "life threatening medical emergencies." In these cases, patients or family members are required to notify the plan of ER visits within 24-48 hours

Medical Review:

- It consists of a group of medical professionals like nurses, doctors etc.
- The medical professionals investigate the claim sent to them, for criteria like medically necessary, medical records & injury report (if it is an accident related treatment)
- The assessment of the claim is done as per the state's medical rules and regulations
- The assessment is then sent to the claims department for adjudication or review

Credentialing:

- It is the process of enrolling or enlisting providers with an insurance company
- Providers need to fill the enrollment form and sign an agreement for enrollment
- Providers need to update themselves on any change in the contract on a regular basis with the insurance company

Capitation:

- It is a contract between the insurance company and the provider
- Capitation Payment means a payment the STATE makes periodically to the provider for each Enrollee covered under the Contract for the provision of services regardless of whether the Enrollee receives these services during the period covered by the payment.
- Claims will be denied and denied as capitation and adjusted as payment is already been paid in advance.

Billed Amount:

This is the amount charged by the provider for the services rendered by him. Each and every treatment / procedure/ service has a standard billed amount which is determined by AMA

Approved Amount:

Approved amount also called, as the allowed amount is the actual amount paid against each billed amount by the insurance company. This amount is predetermined by each insurance company and varies from company to company

Paid Amount:

The amounts paid to providers to satisfy the contractual liability of the carrier or plan sponsor. These amounts do not include any member liability for ineligible charges or for deductibles or co-payments.

Health Insurance Processes Related to the Patient

Deductible:

- Deductible is the amount to be paid periodically (monthly, bimonthly, quarterly, half yearly or annually) apart from the premium by the insured to the carrier
- Services are reimbursed only if this amount is paid for the given period in which the service is rendered

Co Pay:

- Co pay is a pre-fixed amount charged to the patients for every service provided
- This amount ranges approximately between \$5 to \$25 or more.
- All HMO policy holders are responsible to pay Copay

Maximum benefits:

- It is the maximum benefits or coverage given by an insurance company to the subscriber for his service.
- Insurance companies will deny the claim if the benefits have exceeded the maximum allowed.

Federal Forms



Federal forms are basically known as HCFA forms and HCFA stands for The Health Care Financing Administration.

There are two major forms used in MEDICAL BILLING.

1. HCFA 1500

The HCFA 1500 forms are designed for providers to file a medical claim with the patient's insurance carrier to claim professional charges. This form consists of 33 slots.

The image shows a scan of the HCFA 1500 Health Insurance Claim Form. The form is divided into several sections:

- Section 1: Patient Information** (Top left)
- Section 2: Insurance Information** (Top right)
- Section 3: Health Insurance Claim Form** (Middle left)
- Section 4: Health Insurance Claim Form** (Middle right)
- Section 5: Health Insurance Claim Form** (Bottom left)
- Section 6: Health Insurance Claim Form** (Bottom right)

The form includes a large grid for coding procedures, with columns for ICD-9-CM codes and CPT codes. The grid is organized into rows and columns, with specific instructions for coding.

2. UB 92 FORMS or HCFA 1450

The UB-92 forms are designed for hospitals, nursing homes, and clinics to file a medical claim with the patient's insurance carrier to claim all technical charges (non professional). This form consists of 86 slots.

[illegible]

HCFA 1500 FILING INSTRUCTIONS

Block 1-13 - Patient and Insured Information.

Block 1. Enter the type of health insurance coverage applicable to the claim by checking the appropriate box, e.g., to file a Medicare claim check the Medicare box.

Block 1a. Enter patient's Medicare Health Insurance Claim Number (HICN) irrespective of whether Medicare is the primary or secondary payer.

Block 2. Enter patient's last name, first name, and middle initial, if any, as shown on the patient's insurance card.

Block 3. Enter the patient's date of birth and gender.

Block 4. In case of any other insurance being primary other than Medicare, either through the patient or spouse's employment or any other source, list the name of the insured. If the insured and the patient are the same, enter the word SAME. If Medicare is primary, leave blank.

Block 5. Enter patient's mailing address and telephone number. On the first line enter the street address; the second line, the city and state; the third line, the ZIP code and phone number.

Block 6. Check the appropriate box for patient's relationship to insured after completing Block 4.

Block 7. Enter the insured's address and telephone number. When the address is the same as the patient's, enter the word "SAME."

Complete this Block only after Blocks 4 & 11 are completed.

Block 8. Check the appropriate box for the patient's marital status and whether employed or a student.

Block 9. Enter the last name, first name, and middle initial of the enrollee in a Medigap policy, if it is different from that shown in Block 2. Otherwise, enter the word SAME. If no Medigap benefits are assigned, leave blank. This field may be used in the future for supplemental insurance plans.

Block 9a. Enter the policy and/or group number of the Medigap insurer preceded by Medigap

Block 9b. Enter the Medigap insurer's date of birth and gender.

Block 9c. Leave blank if a Medigap *PAYERID is entered in Block 9d. Otherwise, enter the claims processing address of the Medigap insurer. Use an abbreviated street address, two-letter postal code, and zip code copied from the Medigap insurer's Medigap identification card.

For example:

1257 Anywhere Street

Baltimore, Maryland 21204

Is shown as "1257 Anywhere St MD 21204."

Block 9d. Enter the nine-digit PAYERID number of the Medigap insurer. If no PAYERID number exists, then enter the Medigap insurance program or plan name.

Block 10a Check "YES" or "NO" to indicate whether employment, auto liability, or thru other accidents applies to one or more of the services described.

Block 10c. In Block 24. Enter the state postal code. Any Block checked "YES," indicates the existence of another insurance primary to Medicare. Identify primary insurance information in Block 11.

Block 10d. Use this Block exclusively for Medicaid (MCD) information. If the patient is entitled to Medicaid, enter the patient's Medicaid number preceded by "MCD."

Block 11. THIS BLOCK MUST BE COMPLETED. BY COMPLETING THIS BLOCK, THE PHYSICIAN/SUPPLIER ACKNOWLEDGES HAVING MADE A GOOD FAITH EFFORT TO DETERMINE WHETHER MEDICARE IS THE PRIMARY OR SECONDARY PAYER.

If there is an insurance primary to Medicare, enter the insured's policy or group number and proceed to Blocks 11a - 11c.

Block 11a. Enter the insured's date of birth and gender if different from Block 3.

Block 11b. Enter employer's name, if applicable. If there is a change in the insured's insurance status, e.g., retired, enter the retirement date preceded by the word "RETIRED."

Block 11c. Enter the nine-digit PAYERID number of the primary insurer. If no PAYERID number exists, then enter the complete primary payer's program or plan name. If the primary payer's EOB does not contain the claims processing address, record the primary payer's claims processing address directly on the EOB.

Block 11d. Leave blank. Not required by Medicare.

Block 12. The patient or authorized representative must sign and date this Block unless the signature is on file. In lieu of signing the claim, the patient may sign a statement to be retained in the provider, physician, or supplier file in accordance with §§3047.7-3047.3. If the patient is physically or mentally unable to sign, a representative specified in §3008 may sign on the patient's behalf. In this event, the statement's signature line must indicate the patient's name followed by "by" the representative's name, address, relationship to the patient, and the reason the patient cannot sign. The authorization is effective indefinitely unless patient or the patient's representative revokes this arrangement. The patient's signature authorizes release of medical information necessary to process the claim. It also authorizes payment of benefits to the provider of service or supplier, when the provider of service or supplier accepts assignment on the claim.

Signature by Mark (X) - When an illiterate or physically handicapped enrollee signs by a mark, a witness must enter his/her name and address next to the mark.

Block 13. The signature in this Block authorizes payment of mandated Medigap benefits to the participating physician or supplier. If required Medigap information is included in Block 9 and its subdivisions. The patient or his/her authorized representative signs this Block, or the signature must be on file as a separate Medigap authorization. The Medigap assignment on file in the participating provider of service / supplier's office must be insurer specific. It may state that the authorization applies to all occasions of service until it is revoked.

Blocks 14-33 - Provider of Service or Supplier Information

Block 14. Enter the date of current illness, injury, or pregnancy. For chiropractic services, enter the date of the initiation of the course of treatment and enter the X-ray date in Block 19.

Block 15. Leave blank. Not required by Medicare.

Block 16. Enter date from which patient is employed and unable to work in current occupation. An entry in this field may indicate employment related insurance coverage.

Block 17. Enter the name of the referring or ordering physician if the service or item was ordered or referred by a physician. Referring Physician: A physician who requests an item or service for the beneficiary for which payment may be made under the Medicare Program. Ordering Physician: A physician who orders non-physician services for the patient such as diagnostic laboratory tests, clinical laboratory tests, pharmaceutical services, or durable medical equipment.

Block 17a. Enter Nation provider identifier number of the referring/ordering physician listed in Block 17.

Block 18. Complete this Block when a medical service is furnished as a result of, or subsequent to, a related hospitalization.

Block 19. Enter the date the patient was last seen and the NPI of his/her attending physician when he/she is an independent physical or occupational therapist or physician providing routine foot care.

Block 20. Complete this Block when billing for diagnostic tests subject to purchase price limitations

Block 21. Enter the patient's diagnosis/condition.

Block 22. Leave blank. Not required by Medicare.

Block 23. Enter prior authorization number

Block 24a. Enter the month, day and year for each procedure, service, or supply. When "from" and "to" dates are shown for a series of identical services, enter the number of days or units in column G.

Block 24b. Enter the appropriate place of service

Block 24c. Medicare providers are not required to complete this Block.

Block 24d. Enter the procedures, services or supplies.

Block 24e. Enter the diagnosis code reference number as shown in Block 21, to relate the date of service and the procedures performed to the primary diagnosis. Enter only one reference number per line Block. When multiple services are performed, enter the primary reference number for each service a 1, 2, 3, or 4.

If a situation arises where two or more diagnoses are required for a procedure code (e.g. Pap Smears), you must reference only one of the diagnoses in Block 21.

Block 24f. Enter the charge for each listed service.

Block 24g. Enter the number of days or units. This field is most commonly used for multiple visits, units of supplies, or anesthesia minutes. If only one service is performed, the numeral "1" must be entered.

Block 24h. Leave blank. Not required by Medicare.

Block 24i. Leave blank. Not required by Medicare.

Block 24j. Enter the NPI {national provider identifier} of the performing provider of service/supply if they are a member of a group practice.

Block 24k. NOTE: Enter the first two digits of the NPI in Block 24j. Enter the remaining six digits of the NPI in Block 24k,

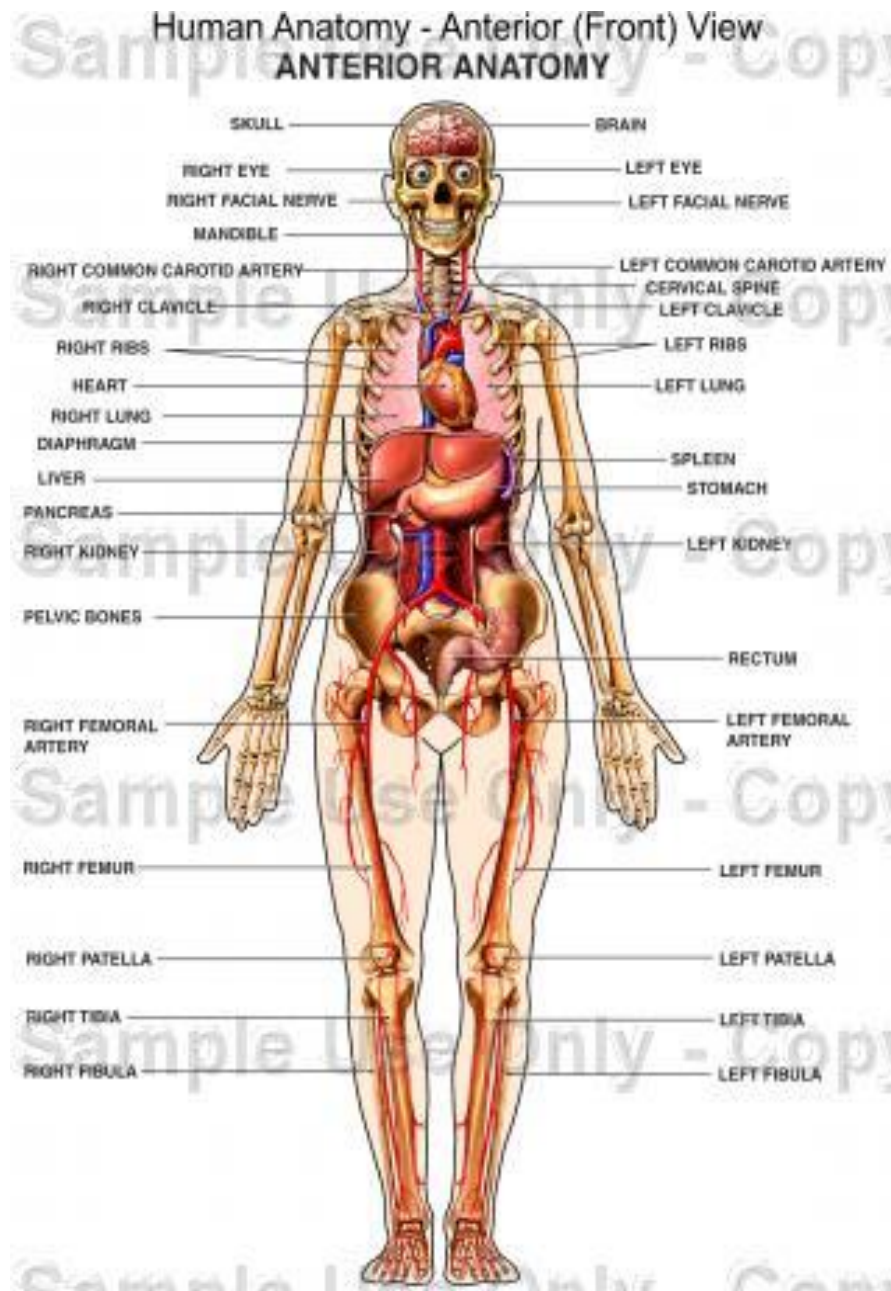
Block 25. Enter your provider of service or supplier, Federal Tax I.D. (Employer Identification Number) or Social Security Number. The participating provider of service or supplier Federal Tax I.D., Number is required for a mandated Medigap transfer.

Block 26. Enter the patient's account number assigned by the provider of service or supply.

Block 27. Check the appropriate block to indicate whether the provider of service or supplier accepts assignment of Medicare benefits.

Block 33. Enter the provider of service/supplier's billing name, address, zip code, and telephone number.

UB92 FORM



Departments of Medicine

1. Emergency Medicine:

- The branch of medicine that deals with evaluation and initial treatment of medical conditions caused by trauma or sudden illness.



2. Pathology:

- The branch of medical science that studies the causes, nature and effects of diseases.



3. Radiology:

- The science of diagnosis and/or treatment using radiant energy. Includes X rays, CT scan, and destruction of tumors by radiation.

- A radiologist uses x-rays, radiant dyes, nuclear medicine, ultrasound, and magnetic resonance imaging to "photograph" the skeletal, arterial and soft tissue areas of the body to diagnose and treat disease or injury



4. Surgery:

- The art of healing by manual operation.
- That branch of medical science, which treats of manual operations for the healing of diseases or injuries of the body.



5. Cardiology:

- The medical study of the structure, function, and disorders of the heart



6. Internal Medicine:

- The branch of medicine that deals with the diagnosis and nonsurgical treatment of diseases affecting the internal organs of the body, especially in adults.



Endoscopy

7. Family Medicine:

- The branch of medicine that deals with provision of comprehensive health care to people regardless of age or sex while placing particular emphasis on the family unit. Also called family practice.



8. Gastroenterology:

- The branch of medicine dealing with the study of disorders affecting the stomach, intestines, and associated organs



9. Oncology:

- The branch of medicine that deals with tumors, including study of their development, diagnosis, treatment, and prevention



MEDICAL DOCUMENTATION:

Medical Record

The physician may use any of a variety of systems to record the medical data of patients. The patient's *chart, record, or file* is the accumulation of all data pertaining to that patient and will include any of the following documents:

- ◆ History and physical examination.
- ◆ Chart notes made by the doctor or nurse.
- ◆ Laboratory and X-ray reports.
- ◆ Special procedure reports.
- ◆ Correspondence.
- ◆ Forms used for a specific purpose, such as an immunization record, developmental and growth records for children, preemployment physicals, preoperative physicals, disability reports, and burn or injury diagrams.

Clinical Data Record

There is an entry or a notation made each time the patient is seen in the office. Entries include the date of the visit and the name of the person who saw or examined the patient.

Narrative Notes

The notation begins with a statement about why the patient is seeking the physician's advice. This reason for the visit may be stated as a symptom or sign and may be referred to as the *chief complaint (CC)* or *problem*.

A complete physical examination is very detailed and follows this specific format:

HISTORY OF PRESENT ILLNESS (HPI), HISTORY, or SUBJECTIVE is information given by the patient. This information includes symptoms, when they began, associated factors, and remedies tried.

PAST MEDICAL HISTORY (PMH) includes illnesses, injuries, and surgeries the patient may have had as well as any allergies to medications or to other substances.

FAMILY HISTORY (FH) consists of facts about the health of the patient's blood relatives that might be significant to the patient's condition.

SOCIAL HISTORY (SH) and **MARITAL HISTORY** are included if pertinent to the patient's treatment. It may include eating, drinking, and smoking habits, as well as occupation and interests of the patient.

REVIEW OF SYSTEMS (ROS) includes a review of each body system, with the physician asking specific questions about the functioning of each system.

OBJECTIVE or PHYSICAL EXAMINATION (PE) provides an examination record. Laboratory and X-ray findings are also considered objective information, although they are usually placed in a separate paragraph.

DIAGNOSIS, ASSESSMENT, IMPRESSION, APPRAISAL, or CONCLUSION provides the examining physician's interpretation of the information.

This may need further studying, in which case a diagnosis may be referred to as *rule out (R/O)*, and further studies will be planned.

PLAN, DISPOSITION, TREATMENT, RECOMMENDATIONS, or ADVICE include instructions to the patient, additional investigational procedures to be performed, medications prescribed, and so forth.

In routine physician office visits, referring to any visit other than a physical exam, the examination is limited to the immediate complaint or symptom. These visits may also be referred to as checkups, rechecks, or progress notes. (*Patient returns to recheck ears. Patient returns for blood pressure check.*)

The sequence of headings for a complete history and physical examination may include the following:

- ◆ CHIEF COMPLAINT
- ◆ HISTORY OF PRESENT ILLNESS
- ◆ PAST MEDICAL HISTORY
- ◆ ALLERGIES
- ◆ MEDICATIONS
- ◆ SURGERIES
- ◆ FAMILY HISTORY
- ◆ SOCIAL HISTORY
- ◆ REVIEW OF SYSTEMS
- ◆ PHYSICAL EXAMINATION
 - GENERAL
 - HEENT
 - NECK
 - CHEST
 - LUNGS
 - HEART
 - ABDOMEN
 - PELVIC
 - RECTAL
 - EXTREMITIES
 - NEUROLOGIC
- ◆ IMPRESSION
- ◆ PLAN

ANATOMICAL COMBINING FORMS

Term	Meaning
adeno	gland
angio	vessel
arterio	artery
arthro	joint
blepharo	eyelid
cardio	heart
cephalo	head
cervico	neck
chole	gall
chondro	cartilage
costo	ribs
cranio	cranium, skull
cysto	bladder, sac
cyto	cell
dermo, dermat	skin
encephalo	brain
gastro	stomach
hemo, hemato	blood
hepato	liver
hystero, utero	uterus
leuko	white
lipo	fat
litho	stone
metro	uterine tissue
myelo	bone marrow, spinal cord
myo	muscle
nephro, reno	kidney
neuro	nerve

oophoro
ophthalmo
osteo

pharyngo
pneumono, pulmono
pyelo
pyo
rhino/naso
salpingo
spleno
thoraco
thrombo
uretero
urethro

ovary
eye
bone

pharynx, throat
lungs
pelvis of kidney
pus
nasal tissue, nose
fallopian tubes
spleen
thorax, chest
clot
ureter
urethra

Medical Coding

There are 2 types of Medical codes:

- CPT CODES
- ICD CODES

CPT CODES

CPT Coding Fundamentals

CPT is an acronym for Current Procedural Terminology. Physician's Current Procedural Terminology, Fourth Edition, known as CPT-4 or more commonly CP1, is a systematic listing of codes and descriptions which classify medical services and procedures. CPT codes are used by physicians, hospitals, and other health care professionals, to report specific medical, surgical and diagnostic services and procedures for statistical and third party payment purposes.

The CPT coding system is maintained by the American Medical Association (AMA) and a revised edition of the CPT book is published each fall. The new CPT codes become effective on January 1 st of the following year. The revisions in each new edition are prepared by the CPT Editorial Panel with the assistance of physicians representing all specialties of medicine.

A thorough understanding of the CPT coding system is essential in order to accurately report medical services and procedures, maximize payments from third parties, minimize denials, rejections and reductions from third parties, and to protect the medical practice from audit liability.

Key Points of the CPT Coding System

A provider or coder using the CPT coding system for coding, first chooses the name and associated code of the procedure or service which most accurately identifies and describes the service(s) performed. The provider or coder then chooses names and codes for additional services or procedures. If necessary, modifiers are chosen and added to the selected service and procedure codes. All services or procedures coded must also be documented in the patient's medical record.

According to CPT 2002, The listing of a service or procedure and its CPT code number in a specific section of the CPT coding system does not restrict its use to a specific specialty or group. Any procedure or service in any section of the CPT coding system may be used to report the services rendered by any qualified physician or other health care professional."

The codes and descriptions listed in the CPT coding system are those that are generally consistent with contemporary medical practice and being performed by health care professionals in clinical practice. Inclusion in the CPT coding system does not represent endorsement by the American Medical Association of any particular diagnostic or therapeutic procedure. In addition, inclusion or exclusion of a procedure does not imply any health insurance coverage or reimbursement policy.

- CPT codes describe medical procedures, services and supplies
- All CPT codes are five digit codes
- CPT codes are mandated by federal law for Medicare, Medicaid, CHAMPUS and Federal Employee Health Plan (FE HP) reporting and are accepted or required by all other third party payers
- CPT codes are self-definitive, with the exception of CPT codes for unlisted procedures and or the few CPT codes which include the term specify, in the description
- Each CPT code number represents the universal definition of a service or procedure
- CPT codes are revised and updated annually by the AMA and the revisions become effective on the 1 st of January every year. Hundreds of CPT codes are added, changed or deleted each year. All health care professionals, third party payers, and health care facilities must maintain copies of the current code books.
- Accurate CPT coding provides an efficient method of communicating medical, surgical and diagnostic services and procedures among health care professionals, health care facilities, and third party payers.
- Accurate CPT coding enhances the health care provider's control of the reimbursement process.

Structure of the CPT Coding System

The CPT coding system includes over 7,900 codes and definitions for medical services, procedures and diagnostic tests. Each procedure or service is identified by a five digit code, followed by the definition.

The CPT coding system is divided into seven Sections. The seven sections of are:

- | | |
|-------------------------------------|-------------|
| • Evaluation and Management - | 99200-99499 |
| • Anesthesiology - | 00100-01999 |
| • Surgery - | 10000-69999 |
| • Radiology - | 70000-79999 |
| • Pathology and Laboratory - | 80000-89999 |
| • Medicine - | 90000-99199 |
| • Category III Emerging Technology- | 0001T-0026T |

Each section of the CPT book includes subsections with anatomic, procedural, condition, or descriptor subheadings. The Evaluation and Management (E/M) section is presented first because

- 1) These codes are used by virtually all health care providers
- 2) They are the most frequently used CPT codes.

ICD CODES

ICD Codes are the International Classification Of Diseases codes or Diagnosis Codes.

ICD-9-CM is an acronym for International Classification of Diseases, 9th Revision, Clinical Modification, published under different names since 1900. ICD-9-CM is a statistical classification system that arranges diseases and injuries into groups according to established criteria. Most ICD-9-CM codes are numeric and consist of three, four or five numbers and a description. The codes are revised approximately every 10 years by the World Health Organization and annual updates are published by the Centers for Medicare and Medicaid Services (CMS).

The concept of extending the International Classification of Diseases for use in hospital indexing was originally developed in response to a need for a more efficient basis for storage and retrieval of diagnostic data. In 1950, the U.S. Public Health Service and the Veterans Administration began independent tests of the International Classification of Diseases for hospital indexing purposes.

In view of the growing interest in the use of the International Classification of Diseases for hospital indexing, a study was undertaken in 1956 by the American Medical Association and the American Medical Record Association of the relative efficiencies of coding systems for diagnostic indexing. Following this study, the major uses of the International Classification of Diseases for hospital indexing purposes consolidated their experiences and an adaptation was published in December 1959. A revision containing the first "Classification of Operations and Treatments" was published in 1962.

Note: ICD Codes are available online software.

MODIFIERS

Modifiers can be classified into two types:

- CPT Modifiers
- Healthcare common procedural coding system (HCPCS)

CPT Modifiers

The CPT coding system includes two-digit modifier codes which are used to report that a service or procedure has been "altered or modified by some specific circumstance" without altering or modifying the basic definition or CPT code.

The proper use of CPT modifiers can speed up claim processing and increase reimbursement, while the improper use of CPT modifiers may result in claim delays or claim denials. In addition, using certain CPT modifiers, for example -22, too often may trigger a claims audit.

Examples of a few most commonly used CPT Modifiers :

- 21 - Prolonged evaluation and management services.
- 22- Unusual Procedural Services
- 25- Significant, individually identifiable evaluation and management service by the same physician on the same day of a procedure or service.
- 26- Professional component.
- 50- Bilateral Procedures
- 51- Multiple Procedures
- 52— Reduced services.
- 57— Decision for surgery
- 62- Two Surgeons
- 76— Repeat procedure by same physician.
- 80— Assistant surgeons
- 99— Multiple modifiers

HCPCS Modifiers

HCPCS modifiers are defined and managed by the centers for Medicare and Medicaid (CMS). Formerly known as HCFA, they are two digit modifiers / codes which alpha or alphanumeric. HCPCS modifiers are used to modify procedures and services on health insurance claim forms filed for Medicare patients.

HCPCS modifiers may be added,deleted or changed in each new edition of HCPCS, therefore, providers should acquire and review the most current edition of HCPCS each year.HCPCS modifiers may be LEVEL II,which are used nationally or LEVEL III, which are used only by local Medicare intermediary.

HCPCS LEVEL II MODIFIERS:

-AA

Anesthesia services by anesthesiologist.

-AR

Ambulance return trip.

-E1

Upper left, eyelid.

-E4

Lower right, eyelid.

-Q6

Service furnished by a Locum Tenens physician.

PLACE OF SERVICE CODES

Listed below are the POS codes and descriptions. These codes should be used on preofessional claims to specify the entity where services were rendered.

11 - Office

Location, other than a hospital, skilled nursing facility (SNF), military treatment facility, community health center, State or local public health clinic or intermediate care facility (ICF), where the health professional routinely provides health examinations, diagnosis, and treatment of illness or injury or an ambulatory service.

12 - Home

Location, other than a hospital or other facility, where the patient receives care in a private residence.

21 Inpatient

A Hospital / a facility, other than psychiatric, which primarily provides diagnostic, therapeutic (both surgical and nonsurgical), and rehabilitation services by, or under, the supervision of physicians to patients admitted for a variety of medical conditions.

22 Outpatient

Hospital

A portion of a hospital which provides diagnostic, therapeutic (both surgical and nonsurgical), and rehabilitation services to sick or injured persons who do not require hospitalization or institutionalization.

23 Emergency Room

Hospital

A portion of a hospital where emergency diagnosis and treatment of illness or injury is provided.

24 Ambulatory

Surgical Center

A freestanding facility, other than a physician's office, where surgical and diagnostic services are provided on an ambulatory basis.

31 Skilled Nursing Facilities

A facility, which primarily provides skilled nursing care and related services to patients who require medical, nursing, or rehabilitative services but not the level of care or treatment available in a hospital.

34 Hospice

A facility, other than a patient's home, in which palliative and supportive care for terminally ill patients and their families are provided.

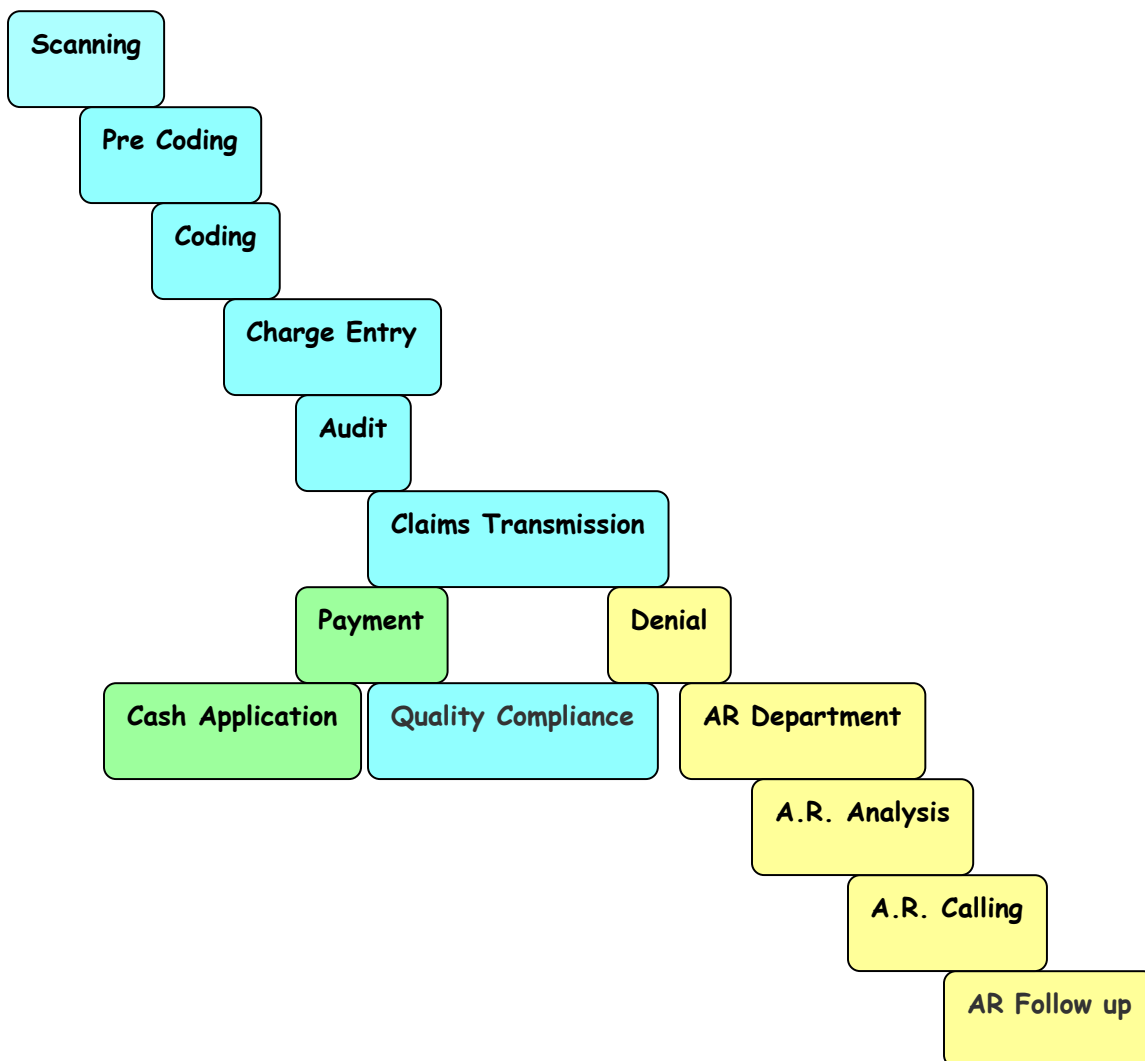
Codes: E & V

E codes denotes Injury

V codes denotes general illness

The Billing Process

Departments in a Billing Office



Step - 1 - Doctor's Office

- Patient visits doctor and explains his/her problem
- Doctor evaluates patients condition
- Determines the diagnoses of the illness / injury
- Draws a treatment plan based on the diagnosis

Step - 2 - Documentation at the Front Desk

- Patient hands over health insurance card copy
- Front office manager verifies patient's insurance status
- Documents patient's demographic details
- Referral or pre-authorization is obtained from patient's Primary Care Physician (Gatekeeper) if mandatory by the insurance company

Step - 3 - Transcription

- The medical records are sent by the Physician's Office to the Medical Transcriptionist
- Medical Transcriptionists convert the voice files (medical records) into data files

Step- 4 - Scanning

- Demographics, charge sheets, super bills, insurance verification data and a copy of the insurance card i.e. all information pertaining to the patient, are sent to the billing office
- All documents are scanned at the Billing office & stored in a central database

Step - 5 - Precoding

- The medical records are sent as data files by the Transcriptionists to the Coders
- Pre-coders enter the key-in codes for
 - Insurance companies
 - Doctors
 - Modifiers

Step - 6 - Medical Coding

- Medical Coders assign the Alpha / Numeric codes.
- The diagnostic records are assigned ICD codes - International Classification of diseases Codes
- The treatment or procedure records are assigned CPT codes - Current Procedural Terminology Codes

Step - 7 - Charge Entry

- Patient's personal information is entered from the Demographic sheets
- All the details of the procedure are entered in two formats. The Charge Sheet & the Super Bill
- The relevance of the CPT codes to the ICD codes is verified.
- A charge / claim is created, in line with the billing rules pertaining to specific carriers and locations.
- All charge entries are accomplished within the agreed turnaround time with the client, which is generally 24 hours.

Step - 8 - Audit

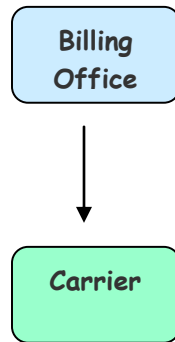


Billing Errors

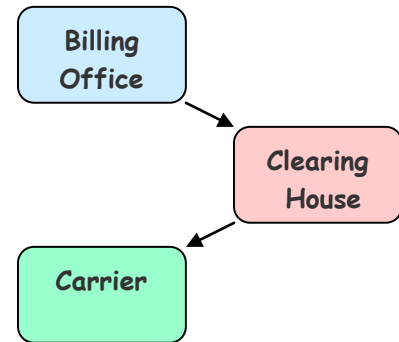
- The daily charge entry is audited to double check the accuracy of the entry / claim, thus ensuring that the billing rule is being followed accurately.
- The verification also ensures the accuracy of the claims, based on the carrier's requirements to ascertain a clean claim.

Step - 9 - Claims Transmission

Direct Transmission



Indirect Transmission



- Claims are filed and information sent to the Claims Transmission department
- Paper claims & Electronic claims are segregated
- Confirmation reports are obtained for claims transmitted electronically, and filed after verification
- Paper claims are printed and attachments done if necessary and mailed
- Transmission rejections are analyzed and appropriate corrective actions are taken

Step - 10 - Carrier Adjudication

- In the Insurance Company the claim is received and kept on file
- The Claims Department, of the insurance company reviews the claim
- On completion of verification by the claims department, the claim is adjudicated or processed for payment.
- An EOB (Explanation of Benefits), which gives details of the reasons for payment or denial, is sent to the provider / billing office.

Step - 11 - Payment - Cash Application



- If the claim is paid, it is then sent to the Cash Applications department of the billing office
- Cash Applications team receives the cash files (Check Copy & EOB) from the insurance company
- The cash or credits received are applied (credited) to the respective patients account (in the billing software)
- During cash application, overpayment's are identified and necessary refund requests generated
- Underpayments & denials are informed to the Analysts

Step - 12 - Denial - AR Analysis

- If a claim is denied or underpaid, it is sent to the AR department of the billing office
- AR Analysts research & analyze the EOB for understanding denial issues
- Work orders are set up for the AR Callers to make calls
- AR analysts are responsible for the cash collections and resolving of all issues so as to enable the account to have a clean AR (accounts receivable zero)
- They also research claims for completeness and accuracy, rejections received from the clearing house, low payment by the carriers and take appropriate actions
- Analysts also review for global patterns and bulk problems

Step - 13 - AR Calling

- Once a work order is set by the analyst for the caller, the caller initiates a call to the Insurance Company and verifies the claim and its current status
- The caller gives inputs to the analysts on the claim status as to whether it is being processed for payment or denial
- Based on these inputs the analyst gets the required pre-requisites
- In case of payment a list of payment details is compiled
- If it's a denial then corrective action is initiated
- All reasons are passed on to the analysts for resolution

Step - 14 - Compilation

- All information pertaining to the patients account is compiled in an Excel Sheet and stored in the data bank for future use
- Global AR patterns are derived from this information

Step - 15 - Month End Reports

- Reports are analyzed every month
- The momentum achieved for the month is determined
- Patterns for non-payment are ascertained & bulk pending issues tackled

Step - 16 - Confidentiality of Information

- Electronic processing and transfer of data via multiplex /router/ modem is encrypted and password protected to ensure privacy and confidentiality
- Dedicated leased lines and Firewalls ensure security of data
- Compliance with The Health Insurance Portability and Accountability Act of 1996 (HIPAA) is ensured
- Confidentiality is maintained for all patient information provided by the client

AR - Account Receivables Issues

Introduction:



Definition: Accounts Receivable may be defined as the total amount of money owed to a practice for services rendered but not yet paid for by the patient or the carrier.

The AR Department is responsible for following up with the patients and carriers on unpaid accounts.

AR issues can be explained as claim denials by a carrier due to inaccurate or incomplete claim processing. It is sent to the billing office in the form of an EOB. It is usually identified by the caller / analyst. The most important criteria is identifying the issue. In order to identify an issue the caller should be well equipped with knowledge of all the issues that may arise in AR Calling.

The provider has open accounts receivable (A/R) cases where the insurance company or patient has been billed and some amount is still outstanding from the insurance or patient. Managing account receivables is nothing but collecting the outstanding amount.

AR ANALYSIS:

Analyzing an account would result in the next stage of adjudicating a call to the carrier. Proper analysis is the first hurdle for an AR Executive. An account needs to be analyzed before the carrier is called. For every claim on file & adjudicated by the carrier, either payment or denial happens. This explanation is given in the form of an EOB. Depending on this, a call is made to the carrier to resolve denials.

AR CALLING:



"It says: 'Last reminder. You haven't yet settled our invoice n° 23002. We look forward to receiving your payment by return, or you won't even begin to know the meaning of the word trouble !'"

After analysis of a denial, the caller calls the carrier. Once a call is initiated the outcome of the call is nothing but identifying the issue. Necessary actions that need to be taken are decided depending on the issue. Hence identifying the issue carries the maximum importance in AR Calling.

Identifying & Solving AR Issues:

AR Issues are identified, addressed and actions taken appropriately.

When a claim is filed with the carrier and the claim is on file at the insurance company (i.e., the claim is in the insurance company's system), the outcome should be either a denial or a payment. If we receive a denial or a low payment or if we do not receive any information from the insurance company within the usual claims processing time, a call is initiated to the carrier to check the status of the filed claim.

Outcome of the call:

The outcome of a call is to identify the issue and take appropriate actions. The issue involves all type of denials, payments and all other outcomes of the call. Each of these issues are handled separately and actions taken appropriately.

1 - CLAIM NOT ON FILE

Issue:

- No response from the carrier on the status of a claim within the usual processing time. (Usually 45 days from the date of submission of claim).
- The claim is not on file or claim not in system

Course of Action:

Step -1

Caller needs to verify the following:

- Claims mailing address: The address to which the claim was sent. The reason for verifying is to identify the correct claims mailing address of the carrier
- Claims filing limit: Filing limit is a time limit within which the carrier should receive the claim. It varies from carrier to carrier.

If claim filed within filing limit, then to attach proof of timely filing. A softcopy (screen shot) of the page showing the original filing details (that the claim was submitted within the filing time limit) to be sent to the carrier.

- If carrier does not accept computer screen shot as proof for timely filing, then to send the Scrubber Report from the clearinghouse
- Claims acceptance thru Fax. If yes, to get the fax number & name of the contact person
- Electronic or Paper: For Claims accepted electronically to get electronic claims payer ID #. (Five-digit number for electronic claims submission)

Step - 2

- To refile the claim after verification of all the above details
- Note: Insurance companies that require NEIC report for electronic submissions are: Blue, PHS

Insurance companies that ask for a certified mail receipt for original submission: Fidelis

All other insurance companies accept computer screen print as proof of timely filing

2- UNTIMELY FILING

Issue:

- Claim filed after filing limit or untimely filing

Course of Action:

Step -1

If claim filed within filing limit, then

- To attach proof of timely filing. A softcopy (screen shot) of the page showing the original filing details (that the claim was submitted within the filing time limit) to be sent to the carrier
- If carrier does not accept computer screen shot as proof for timely filing, then to send the Scrubber Report from the clearinghouse
- To verify claims acceptance thru fax. If yes, to get the fax number & name of the contact person
- For electronic or paper claims, to get electronic claims payer ID #. (Five-digit number for electronic claims submission)
- To refile the claim after verification of all the above details

Step - 2

If claim filed beyond filing limit, then

- To get the claim denial date and the claim #. Carriers deny claims filed after the filing time limit
- To verify whether refiling claim with original filing details is sufficient or an appeal is mandatory
- If an appeal is mandatory, to get appeal address & appeal filing limit. Verify the same with the data available in the provider par information sheet

3 - CLAIM IN PROCESS

Issue:

- The carrier is processing claim. The filed claim is within the claims processing time of the carrier. The status of such a claim is termed as claim in process

Course of Action:

Step -1

- Any additional information to be obtained from the provider, which is necessary for processing the claim, is to be obtained from the provider
- Caller to initiate a call to the provider and obtain the required information
- To send the information electronically or through fax to the carrier

Step - 2

- To send the claim for reprocessing
- To get the date of claim receipt, carrier claim processing time and claim #.
- To request a copy of the EOB

4 - CLAIM PAID

Issue:

- Claim as already paid
- When the carrier processes a claim for payment, the payment is sent by check along with the EOB to the provider's / billing office. If such a payment is not received by either office, then the claim is sent for AR follow-up

Course of Action:

Step -1

- To confirm if the claim has been paid
- To check the following for paid claims: Allowed amount, Paid amount, Co-pay, Co-insurance, Write-off, Check#, Address to which the check has been sent (pay-to address)
- To verify whether the check has been cashed or not

Step - 2

- If check mailed to a different address then to request a copy of the Cancelled check (Front and back) along with the EOB
 - To send a W9 form to the provider's office

5- ELIGIBILITY

Eligibility issue is classified into four different kinds.

(A) Patient cannot be identified as insured:

Issue:

- Patient cannot be identified as insured
- The claim is not on file or claim not in system

Course of Action:

Step -1

Caller needs to verify the following in the given order:

- Patient's SSN, DOB, Last Name, First Name, Policy #, Group # and Address

Step - 2

- If patient is still unidentifiable after verifying the above data then verify that there are no claims paid to this patient by the carrier for a different DOS, from that given
- If there is no other claim paid then all payment or denial details of that claim, to be provided to the carrier to identify the patient
- In there is a claim paid for a different DOS then to mention the same to the carrier

(B) Subscriber's policy terminated:

Issue:

- Subscriber's policy terminated before the date of service.
- The claim is not on file

Course of Action:

- The patient is billed directly

(C) Subscriber's policy not effective on the DOS:

Issue:

- Subscriber's policy not in effect the given DOS
- The claim is not on file

Course of Action:

- The patient is billed directly

(D) Patient not covered under the Carrier's plan:

Issue:

- Subscriber's policy not covered under the carriers plan
- The claim is not on file

Course of Action:

- The patient is billed directly

6 - NON-COVERED SERVICE

Issue:

- The service rendered to the patient is not covered by the patient's insurance plan

Course of Action:

Step -1

- To identify whether the charge is billable to the patient or is a provider write-off, as all claims denied as non-covered services cannot be billed to the patient
- To ascertain the cause of the denial as the reason being:
 - a) That the service is not covered under the patients plan. Then to bill the patient
 - Or
 - b) That the provider cannot bill for that service. Certain procedures are not billable by some providers. Only specialists can bill for them. Then it is a provider's write-off

Step - 2

- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB

7 - CAPITATION

Issue:

- Claim processed towards Capitation

Course of Action:

Step -1

- To confirm if the claim has been processed towards Capitation
- To check the following:
 - Claim #
 - Claim process date
 - Copy of the EOB
 - The allowed amount for the claim

Step - 2

- To suggest the adjustment of the claim

8 – AUTHORIZATION

Issue:

- Claim denied stating No Authorization

Case 2: -

Course of Action:

Step -1

- To confirm if the claim has been denied, stating no authorization
- To check whether authorization is to be obtained by patient or provider
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - Type of plan

Step - 2

- To Check if the services rendered require authorization

Step - 3

- If provider is responsible for getting the authorization
- Check for any claim on file for hospital charges for the same DOS
- If yes, to send claim for reprocessing

Step - 4

- To call the UM dept for authorization details
- If yes, to send claim for reprocessing
- If no send a request to the client for getting the authorization details.
- If patient is responsible for getting the auth
- bill the patient

9 – REFERRAL/ REFERRAL NUMBER of PERMISSIBLE VISITS/ REFERRAL EXPIRED

Issue:

- Claim denied stating No Referral

Course of Action:

Step -1

- To confirm if the claim has been denied, stating no referral
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
 - Provider participating or non-participating information
 - PCP contact details

Step - 2

- Check for any claim on file for hospital charges for the same DOS
- To check with the U&M Department for any referral on file
- If no, to check whether any referral for different date
- If yes, to send claim for reprocessing
- If no, to check whether Back Dating of referral is possible
- If yes, to check the back dating referral period
- If yes, send a request to the client for referral details with PCP information or call the PCP's office for getting referral details.

Back Dating of Referrals

- To call the PCP's office
- To speak with the referral coordinator
- To check for any referral on file for the corresponding patient and DOS
- If yes, then to verify the following details: - Date of Referral, Diagnosis Code, No of permissible visits, Referring and Rendering Physicians names & Validity Dates
- To send the claim for reprocessing
- If no, then request to backdate a referral for the services rendered
- If the referral coordinator is to call back to try thrice before billing the patient on non-receipt of any information from the PCP's office
- If the referral coordinator enquiries about the diagnosis codes and / or any other service details, then to inform that we will call back with the requested information

10 – LACK of PRE-EXISTING INFORMATION

Issue:

- Claim pending / denied stating No Pre-Existing information from the patient / provider

Course of Action:

If Pending:

Step -1

- To confirm if the claim has been processed & kept pending, stating no Pre-Existing information from the patient / provider
- To check the following:
 - Claim #
 - Claim process date
 - Copy of the EOB
 - The type of plan
 - Whether lack of information from patient / provider
- If from Patient:
 - To check the date of mailing the letter to the patient
 - To verify whether patient has responded to the mail
 - If yes, then send the claim for reprocessing
 - If no, any additional information to be obtained from the patient, which is necessary for processing the claim, is to be obtained from the patient
 - Caller to initiate a call to the patient and request him/her to provide the required information to the carrier
 - To call the carrier after 1 month

.

If from Provider:

- To check the name of the provider
- If provider different from our client this information will not be disclosed
- To check the date of mailing the letter to the provider
- To verify whether the provider has responded to the mail
- If yes, then send the claim for reprocessing
- If no, any additional information to be obtained from the provider, which is necessary for processing the claim, is to be obtained from the provider
- Caller to initiate a call to the provider and obtain the required information
- To send the information electronically or thru fax to the carrier
- To call the carrier after 1 month

If Denied: To bill the patient.

11– LACK of COB INFORMATION

Issue:

- Claim pending / denied stating lack of COB information from the patient / provider

Course of Action:

If Pending from Patient:

Step -1

- To confirm if the claim has been processed & kept pending, stating no COB information from the patient
- To check the following:
 - Claim #
 - Claim process date
 - Copy of the EOB
 - The type of plan

Step -2

- To check the date of mailing the letter to the patient
- To verify whether patient has responded to the mail
- If yes, then send the claim for reprocessing
- If no, any additional information to be obtained from the patient, which is necessary for processing the claim, is to be obtained from the patient
- Caller to initiate a call to the patient and request him/her to provide the required information to the carrier
- To call the carrier after 1 month

If Denied: To bill the patient.

12 – NOT A MEDICAL NECESSITY

Issue:

- Claim denied stating not medical necessity

Course of Action:

Step -1

- To confirm if the claim has been denied, stating not a medical necessity
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
 - To check whether the claim will be reviewed on submission of the LOMN or the medical records
 -
 - If yes, then check the appeal's address or the fax # for claims submission
 - If no, it is a provider's write-off

13 - MAXIMUM BENEFITS EXHAUSTED

Issue:

- Claim denied as maximum benefits assigned for the patient exhausted. Maximum benefits are a predefined amount, which is set as the maximum amount that will be covered by the carrier
- The insured & the carrier in the contract agree upon this amount. Amounts exceeding the maximum benefit amount will be denied by the carrier

Course of Action:

Step -1

- To confirm if the claim exceeds the maximum benefit amount

Step - 2

- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
- To bill the patient if the maximum benefit amount has been exceeded

14 - DIAGNOSIS & PROCEDURE CODES MISMATCH

Issue:

- The procedure or treatment does not correspond to the diagnosis. In other words the procedure codes & the diagnosis codes do not match

Course of Action:

Step -1

- To confirm if the diagnosis & procedure codes do not match
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
 - To check whether the claim will be reviewed on submission after correction of the codes
 -
 - If yes, then check the appeal's address or the fax # for claims submission
- To send additional medical documentation such as medical records or any supporting document for the procedure / service rendered

Step - 2

- To send the claim for reprocessing after correcting the procedure & diagnosis codes

15 - DEDUCTIBLE

Issue:

- The claim processed as paid to deductible. This means that the claim is not denied, but adjusted against the deductible amount as the patient has not paid his deductible due for that period of service

Course of Action:

Step -1

- To confirm if the claim is processed towards deductible
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan

Step - 2

- Deductible is a predefined amount to be paid periodically (monthly, bimonthly, quarterly, half yearly or annually) apart from the premium by the insured to the carrier
- Services are reimbursed only if this amount is paid for the given period in which the service is rendered
- To ascertain whether the patient has paid the deductible amount due
- If deductible is not paid, the carrier will bill the patient for that amount & pay the same to the provider
- If it is out-of network, then to bill the patient for the difference between the billed amount and the allowed amount
- If in network it is a provider's write-off

16 - DUPLICATION

Issue:

- The claim is a duplicate. Two claims claimed for the same date of service, with the same procedure code, same diagnosis code, same modifier/s and billed for the same dollar amount for same services rendered by the same physician are termed as duplicate claims

Course of Action:

Step -1

- To confirm if the claim is denied for duplication
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan

Step - 2

For Professional Claims:

To verify the following:

- The date of service, the procedure code, the diagnosis code, the provider name and the modifier (if any)
- If any of the above verified details is different from the details of the denied claim, then the claim is not a duplicate
- If claim not duplicate then to send the claim for reprocessing (with modifier 76 for the same provider & 77 for a different provider)

For Hospital Claims:

- To check the status of the originally processed claim

17 - OUT of POCKET EXPENSES

Issue:

- The claim processed as out of pocket expenses

Course of Action:

Step -1

- To confirm if the claim is processed towards out of pocket expenses
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
 - Patients annual out of pocket expenses
 - Total amount including the respective claim for the year in the DOS

Step - 2

- If it is out-of network, then to bill the patient for the difference between the billed amount and the allowed amount
- If in network it is a provider's write-off

18 - PROVIDER NON-PAR

Issue:

- Claim denied stating provider non-par

Course of Action:

Step -1

- To confirm that the claim is denied, stating provider non-par
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
 - If referral or authorization is required. If any of these are required, proceed with AR issue mentioned in each of these categories

Step - 2

- To check the allowed amount
- To bill the patient if the provider is non-par

19 - PRIMARY PAID MAXIMUM

Issue:

- Claim denied by the secondary insurance stating primary carrier paid maximum amount

Course of Action:

Step -1

- To confirm that the claim is denied, stating primary carrier paid maximum amount
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan

Step - 2

- To check the secondary insurance's allowed amount for the services billed
- To check the primary insurance's allowed amount as mentioned in the primary EOB
- If it is greater, then to bill the patient
- If not, to send the claim for reprocessing

20 - PRIMARY EOB

Issue:

- Claim denied stating that it is a primary EOB

Course of Action:

Step -1

- To confirm that the claim is denied, stating it is a primary EOB
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan

Step - 2

- Confirm whether the carrier is the secondary insurance to the patient
- To check the fax # where we can fax the primary EOB [if available] and to whose attention
- Or the claims mailing address

21 - WORK RELATED INJURY

Issue:

- Claim denied stating that it is a work related injury

Course of Action:

Step -1

- To confirm that the claim is denied, stating it is a work related injury by confirming the diagnosis code
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan

Step -2

- If it is an E code then to refile the claim to WC insurance
- If not, to bill the patient
- Or to send the claim for reprocessing stating it is not a Work related injury

22 - RECOUPMENT

Issue:

- Claim denied stating that it is a recoupment

Course of Action:

Step -1

- To confirm that the claim is denied, stating it is a recoupment
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan

Step -2

- To verify the reason for recoupment & the recoupment amount

23 - INCLUSIVE

Issue:

- Claim denied as inclusive. Inclusive means payment for a procedure is included in the primary procedure that is being paid
- When there are two or more procedures that are performed on the same day in the same location, and charges submitted for both procedures, the primary procedure may be paid and the other procedure may be denied stating that the charges for the second procedure is inclusive in the primary procedure

Course of Action:

Step -1

- To confirm if the claim is inclusive
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
- Determine the most appropriate codes for the 2nd procedure
- Analyze the chances for payment or higher amount by the use of a modifier or including charges for the 2nd procedure with the 1st

Step - 2

- To check the primary procedure code to which the procedure code mentioned in the claim form has been included
- To check whether the claim will be reviewed on submission of LOMN or the medical records to substantiate that the services are not inclusive
- If yes, then to get the appeal's address or the fax # for submitting claims

- If no, it will be a provider's write off

24 - BUNDLED

Issue:

- Claim denied as bundled. Bundled means carrier has paid for one of the two similar procedures done at the same time and payment for the 2nd procedure is bundled together with the 1st
- When two or more, similar procedures performed on the same day are billed together, one procedure may be denied as bundled with the 1st i.e., payment of the second procedure is bundled or paid along with the 1st procedure

Course of Action:

Step -1

- To confirm if the claim is bundled
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
- Determine whether the procedures are related and a part of each other
- Analyze and research for a single CPT code describing all the 3 procedures

Step - 2

- To send the claim for reprocessing with the appropriate bundled CPT code, rather than individual CPT codes

Step - 3 - Unbundling

- To unbundle the claims if the procedures are unrelated
- Unbundling is the process of proving that 2 procedures done on the same day and time are unrelated & both procedures are to be paid for separately
- Determine whether one of the procedures have a different diagnosis
- Appeal the second procedure

25 - OFFSET

Issue:

- Claim denied as offset

Course of Action:

Step -1

- To confirm if the claim is offset
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan
 - Reason for offset
 - Offset date
 - Offset amount in \$
 - Offset done to which claim - and it's detail
 - DOS
 - Billed amount in \$
 - Patient's name
 -

26 - GLOBAL

Issue:

- Claim denied as Global. Global means payment for Professional charges are included with the payment for Hospital charges
- When charges for the Provider's service are denied stating that it was paid to the hospital along with the payment for hospital charges, the issue is termed as Global.

Course of Action:

Step -1

- To confirm if the claim is global
- To check the following:
 - Claim #
 - Claim process date
 - Claim denial date
 - Copy of the EOB
 - The type of plan

Step - 2

- Determine whether the professional charges have been paid along with the hospital charges
- If yes, it is a provider write off

HIPAA

HIPAA. The American Health Insurance Portability and Accountability Act of 1996 (HIPAA) is a set of rules to be followed by health plans, doctors, hospitals and other health care providers. HIPAA took effect on April 14, 2003. In the health care and medical profession, the great challenge that HIPAA has created is the assurance that all patient account handling, billing, and medical records are HIPAA compliant.

- Patients must be able access their record and correct errors
- Patients must be informed of how their personal information will be used.
- Patient information can only be shared if needed to treat the patient. In particular, it cannot be used for marketing purposes without their explicit consent.
- Patients can ask their health plans and providers to take reasonable steps to ensure that their communications with the patient are confidential. For instance, a patient can ask to be called on his work number, instead of home or cell phone number.
- Patients can file formal privacy-related complaints to the HHS' Office for Civil Rights.
- Health plans or providers must document their privacy procedures, but they have a lot of freedom on what to include in their privacy procedure.
- Health plans or providers must designate a privacy officer and train their employees.
- Health plans and providers must use standard formats for electronic data interchange, such as electronic claims submission [EDI](#).

Billing Terminologies

1. Term **ADJUDICATION**

Definition/Description

Adjudication is the process of processing claims for either payment or denial by the claims department of the insurance company.

2. Term **A M A**

Definition/Description

AMA stands for American Medical Association. It is an organization of physicians. The AMA is responsible for determining and revising the CPT + ICD codes

3. Term **APPEAL**

Definition/Description

It is a Written or Verbal statement made by the insured to the insurer requesting for reconsidering the initial determination of a claim

4. Term **APPROVED AMOUNT**

Definition/Description

Approved amount also called as the allowed amount is the actual amount paid against each billed amount by the insurance company. This amount is predetermined by each insurance company and varies from company to company

5. Term **ASSIGNMENT OF BENEFITS**

Definition/Description

It is an authorization given by the patient to the doctor to release confidential patient information to third parties. The doctors require authorization for claims processing and payment.

6. Term **AUTHORIZATION**

Definition/Description

It is an authorization given to the doctors to render a particular service to the patient. It is given on the basis of the diagnosis for that treatment / procedure. The Utilization Management Dept of the carrier gives it.

7. Term **BILLING COMPANY**

Definition/Description

They are the intermediaries between the service provider and the carrier. They act on behalf of the provider & submit bills/claims to the carrier. They also follow up on the claim status till the carrier settles the claim.

8. Term **BILLED AMOUNT**

Definition/Description

This is the amount charged by the provider for the services rendered by him. Each and every treatment / procedure/ service has a standard billed amount which is determined by AMA.

9. Term **BUDGET**

Definition/Description

It is a payment option through installments. This option is used when paying deductibles or any patient responsibility.

10. Term **BUNDLED**

Definition/Description

Bundled means carrier has paid for one of the two similar procedures done at the same time and payment for the 2nd procedure is bundled together with the 1st. When two or more, similar procedures performed on the same day are billed together, one procedure may be denied as bundled with the 1st i.e., payment of the second procedure is bundled or paid along with the 1st procedure

11. Term **CAPITATION**

Definition/Description

It is a contract between the insurance company and the doctor, to pay the doctor based on the number of patients treated by him or her for a certain period of time. This is an alternative to getting paid for each claim.

12. Term CARRIER

Definition/Description

Insurance is a contract that exists between an organization & a person, to protect the person from the losses or damage, that he/she might incur on account of the particular risk or hazard, which has been stated therein in the contract. Since the insurance company carries the financial risk of the insured, it is also called a carrier.

13. Term CHARGE SHEET

Definition/Description

Charge sheet is a bill for the services provided by the doctor. It contains the following: date of service, place of service, type of service, procedure codes, modifiers, diagnosis codes, patient name, units of service / visits, admission & discharge dates, time, referring physician and rendering physician.

14. Term CLAIM

Definition/Description

The charge or bill, sent to the carrier for the services provided by the doctor & other hospital charges for payment. They are sent as Paper claims or Electronic Claims.

15. Term CO INSURANCE

Definition/Description

It is an agreement whereby the insured agrees to pay a part of the billed amount for services received.

16. Term **COVERED SERVICES**

Definition/Description

Services rendered to patients, which are within the scope of the policy and are reimbursable by the carrier.

17. Term **CPT CODES**

Definition/Description

CPT stands for Current Procedural Terminology. They are a set of codes developed to define/describe every conceivable medical treatment or procedure. This coding system is developed by the AMA & revised annually.

18. Term **CREDENTIALING**

Definition/Description

When a doctor is enlisted within the network of a carrier, it means that the carrier has credentialed the doctor. The process is done by means of a W9 form.

19. Term **CROSS OVER**

Definition/Description

When the primary carrier (usually Medicare) processes a claim and then sends the claim for processing by the secondary carrier, it is called cross over.

20. Term DEDUCTIBLE

Definition/Description

A sum of money to be paid periodically (monthly, bimonthly, quarterly, half yearly or annually) apart from the premium by the insured to the carrier. Services are reimbursed only if this amount is paid for the given period in which the service is rendered.

21. Term DEMOGRAPHICS

Definition/Description

The patient's demographics consist of both the patient & the physician details. The patient details include name, SSN, address, date of birth, gender, ID no., consent/waiver form, guarantor details, insurance details, insurance card copy, employer info, patient subscriber relation, policy effective & expiry dates. Physician details include Rendering & Referring physician details, referral form & authorization details.

22. Term DOS (DATE OF SERVICE)

Definition/Description

The date on which the service was rendered to the patient.

23. Term DIAGNOSIS

Definition/Description

It is the process of identifying the cause for an illness/injury/condition by assessing the patient's symptoms, past medical history, other medical tests and counseling.

24. Term **DURABLE MEDICAL EQUIPMENT**

Definition/Description

Purchased or rented items such as hospital beds, iron lungs, oxygen equipment, seat lift equipment, wheelchairs, and other necessary medical equipment prescribed by a health care provider to be used in a patient's home and are covered by Medicare.

25. Term **EGHP**

Definition/Description

Employer Group Health Program - is a health insurance sponsored by the employer of either the patient or his/her spouse.

26. Term **EXCLUSIONS**

Definition/Description

Provisions in the law stating situations or conditions which are not covered by the insurance contract/policy.

27. Term **ELIGIBILITY**

Definition/Description

Eligibility refers to whether a patient is covered for health care by the carrier. The policy gives a clear picture of the eligibility of the person.

28. Term **EOB**

Definition/Description

EOB, which stands for Explanation of benefits, is a form, which states the initial determination of a claim (payment/denial) and the reasons for it, as processed by the carrier. It is usually sent to the

billing office.

29. Term FEE SCHEDULE

Definition/Description

It is a list of services and their respective payable amounts. The payable amount indicates the maximum allowed payment for that particular service. The fee schedule is fixed for each carrier and may vary from carrier to carrier.

30. Term FEE FOR SERVICE

Definition/Description

It is a fee charged to the patient for each & every service rendered by the doctor.

31. Term GLOBAL FEE

Definition/Description

The technical & professional charges of a claim combined together is called Global Fee.

32. Term HCFA

Definition/Description

HCFA stands for Health Care Financial Administration. It is a part of the dept of Human services that operate the Medicare Program. It is now known as CMS - Center for Medicare and Medicaid Studies. All professional charges are sent in the form of a HCFA form to Medicare.

33. Term HOSPITAL BASED PHYSICIAN

Definition/Description

A physician who has a contract with the hospital to provide healthcare services in the hospital.

34. Term **IN-PATIENT**

Definition/Description

A patient who stays and uses the hospitals regular / intensive/ institutional facilities & is admitted in the hospital or health facility is called an In-patient.

35. Term **INCLUSIVE**

Definition/Description

Inclusive means payment for a procedure is included in the primary procedure that is being paid. When there are two or more procedures that are performed on the same day in the same location, and charges are submitted for both procedures, sometimes the primary procedure will be paid and the other procedure is denied stating that the second procedure is inclusive with the primary procedure.

36. Term **LINE ITEM**

Definition/Description

It is an item or service specifically detailed in the claim.

37. Term **MEDICAL REVIEW**

Definition/Description

It is a check done to ensure the relevance and authenticity of all procedures/treatment rendered. Medical review ensures that all services rendered are within the scope of the policy, thus ensuring a clean claim & payment.

38. Term MEDICAL NECESSITY

Definition/Description

Services which are necessary & consistent with

- a) The diagnosis and treatment of the insured's condition, disease, ailment or injury.
- b) The standards of good medical practice, which are not provided for the convenience of the insured, the hospital or the physician.
- c) The most appropriate level of service that can be safely provided for that diagnosis or treatment.

39. Term MEDICARE SECONDARY PAYER (MSP)

Definition/Description

A term used to identify Medicare as the Secondary Payer for a health insurance claim. Medicare is identified as Secondary Payer when the insured has medical insurance coverage other than Medicare, which is considered Primary.

40. Term MEDICARE SUMMARY NOTICE

Definition/Description

A form sent by Medicare to the billing office, (provider), detailing how a particular claim has been processed.

41. Term **MODIFIER**

Definition/Description

Modifiers are standard 2 digit alpha or numeric codes, used in conjunction with a CPT or HCPCS codes to alter the original definition / description of that code. Thus modifiers give a new or different meaning to the code. Using modifiers either results in the increase / decrease of the reimbursement amount.

42. Term **NATIONAL PROVIDER IDENTIFIER**

Definition/Description

It is a unique individual identification number assigned to providers. It is also known as Unique Provider Identification Number (UPIN).

43. Term **NON-PARTICIPATING PROVIDER**

Definition/Description

Providers or Suppliers (of health care) who do not participate in insurance programs and are not listed in the network of an insurance company. They are also called as out of network providers.

44. Term **NON-RENDERED SERVICES**

Definition/Description

Services or supplies that are claimed by the provider or supplier without the same being actually provided to the patient.

45. Term **OVER PAYMENT**

Definition/Description

A payment by the carrier, which is greater than the allowed amount for that particular billed

amount, is referred to as overpayment.

46. Term **OUT OF POCKET**

Definition/Description

They are expenses that are the responsibility of the insured and are not reimbursable by the carrier. These are expenses that are not covered by Medicare or any other insurance.

47. Term **PARTICIPATING PROVIDER**

Definition/Description

Providers or Suppliers (of health care) who agree to participate in insurance programs and are listed in the network of an insurance company. They are also called as in network providers. Participatory providers have certain benefits such as, higher allowances, toll free telephone lines for electronic submission of claims, no legal charge limit, medigap benefits and free publicity.

48. Term **PROVIDER / PHYSICIAN / DOCTOR**

Definition/Description

A generic term for any person or entity approved of providing health care to beneficiaries (health insured patients) and is eligible to receive payment for such services rendered, from the carrier. Providers are also referred to as Physicians or Doctors.

49. Term **REFERRAL**

Definition/Description

Referral is a reference from the PCP to consult a specialist for treatment of an ailment / injury

which cannot be attended to by the PCP & needs specialized treatment. Usually the PCP refers a specialist in network.

50. Term RENDERING PHYSICIAN

Definition/Description A physician who treats the patient is called as the rendering physician.

51. Term REFERRING PHYSICIAN

Definition/Description A physician who refers a patient to a specialist or another physician is called as a referring physician. The PCP gives referrals and usually refers a specialist in network.

52. Term REVIEW

Definition/Description The first formal level of appeal, following a denial of a claim is termed as review. Review is a request to the carrier to reconsider a previously determined claim.

53. Term SSN - SOCIAL SECURITY NUMBER

Definition/Description This is a 9-digit number issued in USA, which is a multipurpose identification number. The 9 digits SSN. is divided into 3 parts: The first 3 digits are area numbers. The middle 2 digits are the group numbers. They have no special geographic or data significance but merely serve to break the no. into conveniently sized blocks for orderly issuance. The

last four digits are serial numbers. They represent a straight numeric sequence of digits from 0001 to 9999 within the group. Example: 123-45-6789.

54. Term **SUPER BILL**

Definition/Description

A super bill is like a charge sheet but also contains the insurance information apart from the patient & provider information. A super bill contains the following information in addition to the charge sheet information: charge amount, coverage information, co-pay and patient demographic details.

55. Term **UNBUNDLING**

Definition/Description

Unbundling is the process of proving that 2 procedures done on the same day and time are unrelated & both procedures are to be paid for separately.

56. Term **UP CODING**

Definition/Description

Up coding is a potentially fraudulent activity of submitting false claims to the carrier. False claims are claims submitted for non-covered or non-chargeable services, supplies or equipment. In up coding such claims are sent as charges rendered by a specialist in network.

57. Term **W9 FORM**

Definition/Description

A form which is used by the carrier for provider enrollment in a network.

58. Term	WRITE OFF
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Definition/Description	The difference between the billed and allowed amount is called as provider write off.
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